

THE LONDON MEDICAL GAZETTE,

BEING A
WEEKLY JOURNAL

OF
Medicine and the Collateral Sciences.

SATURDAY, DECEMBER 27, 1828.

PATHOLOGICAL ESSAYS

ON SOME

DISEASES OF THE HEART;

*Being the Substance of Lectures delivered before
the College of Physicians,*

By P. NERE LATHAM, M.D.

Physician to St. Bartholomew's Hospital.

[Continued from p. 7.]

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ESSAY II.

*Morbid Anatomy of the Internal Lining
Membrane of the Heart.*

THE membrane which lines the cavities of the heart is very liable to disease, but not equally so in every part. Where it is thin and transparent, and admits the colour and character of the muscular structure upon which it is spread to be seen through it, it is seldom found diseased; but where it is of a denser texture, either in itself or from an admixture of other structures, whether cellular or fibrous, with its own, it is frequently, and often exclusively diseased. This latter character of a denser texture belongs to it where it forms the tough white circles which surround the apertures of communication between the auricles and ventricles; also where it is reflected upon itself, and forms the loose duplicatures of mem-

just pointed out. Of the fibrous circle between the auricle and ventricle, of the valves which originate from it, and of the tendinous cords which connect the valves with the *carneæ columnæ*, there will not be the smallest space free from disease; but the disease will abruptly stop where the tendinous cords cease and the *carneæ columnæ* begin.

The membrane, however, where it covers the fleshy columns of the heart, is not exempt from the possibility of disease: but when disease actually affects it, it has seldom originated there, but has generally spread from other parts of the same membrane, although (as we have just remarked) it is apt to stop short before it reaches this.

Of the two sides of the heart, the membrane which lines the left is unquestionably the more liable to disease. But my own observation would never have led me to conclude that the membrane of the right side was so far exempt as it is commonly thought to be. Speaking from the best recollection I have of the specimens which have fallen under my examination, I should say that, in one-third of the cases where disease has been found on the left side, it has existed on the right side also, and been essentially of the same character. But there has been a remark-

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The 'Apothecary System' of measurement was used in the original text. The etext uses the following Unicode symbols:

℞	Prescription symbol
℥	Dram
℥	Ounce
℥	Scruple

Roman numerals, and 'ss.' for 'half', are retained, so for example '℥iss.' indicates 'one and a half ounces'.

Obvious typographical errors and punctuation errors have been corrected after careful comparison with other occurrences within the text and consultation of external sources.

More detail can be found at [the end of the book](#).

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It is remarkable how curiously disease is apt to limit itself to the spaces just pointed out. Of the fibrous circle between the auricle and ventricle, of the valves which originate from it, and of the tendinous cords which connect the valves with the carneæ columnæ, there will not be the smallest space free from disease; but the disease will abruptly stop where the tendinous cords cease and the carneæ columnæ begin^[1].

The membrane, however, where it covers the fleshy columns of the heart, is not exempt from the possibility of disease: but when disease actually affects it, it has seldom originated there, but has generally spread from other parts of the same membrane, although (as we have just remarked) it is apt to stop short before it reaches this.

Of the two sides of the heart, the membrane which lines the left is unquestionably the more liable to disease. But my own observation would never have led me to conclude that the membrane of the right side was so far exempt as it is commonly thought to be. Speaking from the best recollection I have of the specimens which have fallen under my examination, I should say that, in one-third of the cases where disease has been found on the left side, it has existed on the right side also, and been essentially of the same character. But there has been a remarkable difference in the extent to which it has proceeded on each side respectively: while on the left it has gone so far as to be the undoubted cause of death, on the right, although essentially of the same character, it has been only just beginning.

It very seldom happens that disease appertains to the lining of the right cavities of the heart exclusively; and, where it affects both, the disease in the right cavities is very seldom found in advance of that in the left.

It should seem, indeed, according to the ordinary course of things, that disease does not begin in the lining of the right cavities of the heart, until it has already advanced to an extreme degree in the left.

The internal lining of the heart, as well as of the arteries, is often found to have become of a red colour. This redness, so well known to all who are accustomed to examine dead bodies, used to be regarded as a mere stain imparted to it by the colouring matter of the blood after death. Yet a due consideration of various circumstances connected with it will hardly warrant this conclusion.

It has been found whether the heart or artery be full or empty of blood; and if blood be present it has been found, whether it is liquid or clotted; and if it be clotted, whether it does or does not retain its colouring matter. External temperature, and length of time between the death of the patient and the dissection of his body, have not made any difference in the frequency with which this peculiar appearance occurs. Lastly, no artificial

methods, such as washing of any kind, can get rid of it; nor will inclosing blood within an artery for any period produce it^[2]. This simple redness is sometimes seen universally in both sides of the heart and throughout the whole arterial system, and sometimes in patches only, of greater or less extent, whether in the arteries or in the heart.

Now, when all these circumstances are considered, although in some instances it may be a mere stain imparted by the colouring matter of the blood after death, it is plainly impossible that it should be of that nature in any large proportion of the numerous instances in which it is found.

But if the appearance in question implies (as I believe it generally does) a morbid condition, of what kind is that condition? There are the same objections to considering mere redness as equivalent to inflammation here as in the pericardium, or in any other part of the body: here, as elsewhere, in one case it may be the condition out of which inflammation is to spring; while in another it may not be destined to give origin to any change in the structure of the part beyond itself, and may itself constitute the whole disease.

It appears to me, that this mere redness of the internal lining of the heart and arteries has become a matter of undue perplexity to pathologists, because they have laboured to infer from it more than the simple fact itself will authorize. All I wish to establish concerning it is, first, that it is not always (probably very seldom) a mere stain imparted by the colouring matter of the blood after death; secondly, that it alone does not constitute inflammation.

It may not be improper to mention the circumstances under which it has occurred to myself to find it. I have met with it most frequently, and to the largest extent, in subjects whose previous disease has produced a constant and habitual impediment to the transmission of blood through the heart and through the lungs, and that impediment has gone on increasing to the hour of their death; also in those, whatever might have been the nature of their disease, whose dissolution (I mean the actual process of dying) has been tardy and agonizing, and marked by great labour of respiration; in the apoplectic, for example, in whom, after sense and consciousness were extinct, life had been protracted, with stertorous breathing, for many days.

In such subjects the countenance, the lips, and the whole skin, give evidence during life of blood pushed beyond the natural sphere of the circulation, and detained in the extreme blood-vessels. Hence it is obvious that the causes which have loaded and distended the capillaries in every part of the body have had a like influence upon the vasa vasorum.

I do not mean to say that I never met with this peculiar condition of the heart and arteries under other circumstances, or that other causes may not produce it; but that I am not acquainted with it under any other with which, from frequent coincidence, it has seemed to have a natural connexion, or which have afforded a reasonable explanation of the phenomenon.

This condition of the heart and arteries, considered as inflammation, has been assigned by some as the cause of fevers of the more malignant kind. The frequency with which it has been found in some particular epidemic, must have led to the conclusion. But, however this may be, from my own observation, not restricting myself to the fever of any particular season, but taking into account all complaints called febrile, and belonging to all seasons, also from the result of inquiry among medical men who have had large acquaintance with morbid dissections, and from the experience of those who have made this particular point a subject of investigation (Laënnec and Andral) I venture to conclude that it has no essential connexion with fevers of any kind, either as cause or as effect.

On some occasions the internal membrane of the heart and arteries, wherever it exhibits the appearance described, will allow itself to be peeled off from the subjacent structure with the least possible force; this facility of separation ceasing entirely beyond the boundary of the red tinge.

Here unquestionably is further evidence of a diseased condition: but of what nature? Most pathologists would consider this to be of the nature of inflammation—and I believe justly.

There is indeed much difficulty in pronouncing upon the nature of minuter changes of structure detected in the internal parts of the body after death. We are obliged to arrive at conclusions by help of analogies drawn from morbid processes, which we have watched in their progress during life, upon the external surfaces; for during life we have the functions and sensibilities of the part to aid us in forming a right judgment concerning its disease. When, during life, one tissue is separated from another, as the

periosteum from the bone, or the cuticle from the skin, or the mutual cohesion between different tissues is sensibly weakened, we find it to be owing to the intervention of serous fluid which does not belong to their healthy state; and this, together with increased vascularity, or redness and heat, and pain, is enough to bespeak the presence of inflammation. All these conditions cannot remain after death. Hence, if we desire to form positive opinions concerning much which is unfolded by dissection, we must supply the defect by analogy. Thus, whenever, in any part of the heart or arteries, the cohesion between the internal membrane and the subjacent structure is manifestly lessened, and the membrane is unusually red at that part, we may regard these appearances as the vestiges of inflammation, without thinking that we go too far in so regarding them.

The internal lining of the heart and arteries is often found red solely in the neighbourhood of ulcerated spaces, when there can be no doubt concerning the existence of inflammatory action.

But the internal lining of the heart and arteries gives the most unequivocal evidence of its inflammation when it is found of a deep red colour, with coagulable lymph adhering to its surface. This condition is represented, as it was found in the aorta, in one of the beautiful plates, illustrative of the diseases of arteries, by Mr. Hodgson^[3]. And the same condition, in the heart, I have seen in a preparation of Dr. Farre's, where lymph is deposited, upon the circular zone, which forms the aperture of communication between the left auricle and ventricle. These appearances denote the most acute inflammation: they are, I suspect, very rarely met with. In the few instances in which I have heard of them, they have been found where death has taken place after short and severe suffering, and with symptoms which characterize inflammation.

But the specimens of disease most frequently met with in the internal lining of the heart, consist in an entire change of its natural structure, and in the formation of new products upon it or within it. Many of these, from the analogy of morbid actions in other parts of the body, must be considered to result from chronic inflammation.

In any of those situations which have been stated as especially liable to disease, the membrane will become thick, tough, inelastic, puckered and shrivelled; and cartilage or gristle, and bone, will enter into its structure;

excrescences will sprout out from it, resembling warts and fungus; and it will become ruptured and ulcerated.

Cartilaginous depositions are often found beneath the membrane where it is single; or between its folds where it is double, in the situation of the valves; and thus they seem rather to belong to some structure contiguous to the membrane than to the membrane itself. Such depositions will proceed to a considerable extent, while the membrane still remains free from disease. From a valve, which has been thick, opaque, and cartilaginous, I have seen the membrane separated on both sides, and transparent; the opaque and cartilaginous matter being left behind. Where, in cartilaginous depositions, the lining of the heart has become puckered and uneven on its surface, and the valves shortened and altered in their shape, the membrane itself participates in the disease, and is generally incapable of being separated from the subjacent structure. But great thickening may take place in the situation of the valves, from deposition of cartilage, without any unevenness of their surface or alteration of their shape; and under these circumstances the membrane itself you may expect to find hitherto exempt from disease^[4].

Osseous depositions are always, I believe, originally formed beneath, or exterior to, the membrane, both in the heart and in the arteries. There are two circumstances especially worthy of remark in this process of ossification: sometimes it is a pure and unmixed process: bone is formed, and nothing else. It is deposited in minute granules, or little brittle scales, or in plates of a larger size; and the intermediate spaces, whether in the heart or arteries, preserve their natural and healthy appearance. At first, these granules or scales, or plates of pure bone, are covered by a delicate pellicle, which is in fact the internal membrane of the heart or artery, separating them from the immediate contact of the circulating blood. But in process of time, as they increase in size, and become rough and unequal on their surface, they cause a rupture of the internal membrane, and have now nothing to separate them from the immediate contact of the blood^[5].

Sometimes ossification is a mixed process, or rather, I suspect, the result of another morbid process preceding it. With the cartilaginous depositions already described there is an admixture of bone. The quantity of bone generally bears a very small proportion to the cartilage when they both occur together, as if the bone proceeded from the cartilage, and not the

cartilage from the bone. It is sometimes seen growing from the surface of the cartilage, and is sometimes deposited in its substance, and only detected by the knife.

Simple ossification, as it occurs in the heart and arteries, has been classed among the natural changes which the parts in question are liable to undergo after a certain period of life. Of persons above the age of sixty years the proportion is that of seven in ten according to Bichât, in which ossification is discovered in some part of the arterial system. It very rarely happens that simple ossification is found before the period of old age: still it is difficult not to regard it as a morbid process.

But ossification, when it is a mixed process, is unquestionably the result of disease. It is met with at all periods of life; and probably constitutes one of the terminations of inflammation. The kind of morbid structures with which the bone is united leads to this belief.

Fungous, and wart-like excrescences, are found in all those parts of the internal lining of the heart, which have been already mentioned as most subject to disease. They seem to be the result of a new morbid action set up in parts already disorganized; for I have not met with them where the membrane has been otherwise healthy, but only where it has been thickened or cartilaginous, or ossified, ruptured, or ulcerated. They grow either from the surface of the membrane, or from its ruptured or ulcerated edges, and are always in immediate contact with the circulating blood^[6].

According to my observation, when a fungous or warty excrescence has grown from the ruptured edges of the membrane, it has been from them exclusively, and from no other part of the lining of the same heart. And as rupture of the membrane seldom occurs in more than one situation at a time, the heart of the same individual seldom presents more than one excrescence of this kind: and as the aortic valves are the parts most liable to rupture, it is there that this single excrescence is most frequently found; not that rupture may not take place elsewhere. I once saw a single *chorda tendinea* ruptured; and a single fungous excrescence of considerable size hanging from it into the cavity of the left ventricle.

Now, this morbid growth is evidently connected with the lacerated state of the membrane in the conditions of its production, whereby it is limited to a small space.

It is probable that the membrane is first ruptured; that its lacerated edges inflame, and then throw out unhealthy lymph, or unhealthy granulations, in the shape of these fungous or wart-like excrescences. When they have sprung from a ruptured membrane, they have, in the specimens which I have examined, been larger than when they have arisen under other conditions.

But when such excrescences grow from the surface of the membrane which is thickened and cartilaginous only, but not lacerated, they are more apt to occur in many parts of it at the same time. I have seen the valvular apparatus between the auricles and ventricles on both sides, as well as the aortic valves of the same heart, studded with them. They were all about the size of hemp seeds: they adhered to the membrane with different degrees of tenacity, and wherever they were capable of being detached, they left a rough surface. The lining of the arteries has been known to give origin to morbid growths of the same kind, which have obstructed the passage of blood, and given occasion to the formation of a coagulum, which has obliterated the pulse.

The internal lining of the heart is liable to ulceration, not as a common consequence of simple inflammation, but as an occasional consequence of some of those diseased conditions which have been described.

It is most commonly found around scales and spiculæ of bone, and under such circumstances as to leave no doubt that the bone itself has furnished the source of irritation from which it springs. The ulceration commences from the very border of the bony scale, as if it was a process of nature for detaching it, and to a considerable distance around the ulceration the membrane is reddened, and easily detached from the subjacent structure. Where there are several distinct scales of bone, it is not uncommon to find a circle of ulceration around each of them.

Ulceration is also met with where there is a thickened and cartilaginous state of the membrane without ossification. Under these circumstances, as far as I know, it affects no definite form. It is often a very destructive process of disease, obliterating large portions of the valvular structure, and penetrating deep into the muscular substance of the heart.

Rupture of the internal lining of the heart is not easily distinguished from ulceration. In collections of morbid anatomy, many unquestionable

specimens of ulceration are described as specimens of rupture. Rupture must always be looked for in the valvular apparatus of the heart, *i. e.* in the valves themselves, or in the chordæ tendineæ which are their appendages. It is probably incapable of taking place elsewhere, except as a part of a rupture, which involves the whole organ.

That solution of continuity which is evidently without loss of substance; that of which the separated edges when they are brought together are completely adapted to each other; also that which is unaccompanied by any thickening or other morbid condition of the valve; the solution of continuity which is found under these circumstances, may safely be considered to proceed from rupture, and not from ulceration^[7].

But it is probable that these characteristic conditions do not long remain after the occurrence of the rupture. Complete specimens of them are very rare; but specimens are numerous where the solution of continuity, by its form and direction, bespeaks rupture, while its rounded edges and the general thickening of the valve denote ulceration. These, it may be fairly conjectured, do in fact exhibit a compound of both. The membrane was originally ruptured, but disease has subsequently arisen and obscured the character of the mechanical injury.

The greater number of those concretions, which were regarded by the older anatomists as polypi of the heart, were unquestionably portions of mere blood, which had undergone coagulation after death. The blood remaining in the heart after death discharges itself of its colouring matter as it coagulates, and, giving off processes between the muscular fasciculi, assumes a shape which has suggested the name of polypus^[8].

I have often found (I presume, therefore, that it is not an uncommon occurrence) coagula of an irregularly laminated texture having their colouring matter not entirely discharged, but unequally distributed through them, which have been most intimately adherent to some part of the lining of the heart. Of these some have admitted of separation, while the surface of the membrane and the surface of the clot were left rough at the place of contact, and others were incapable of being detached without the membrane being detached along with them. The appendix of the left auricle is a situation in which they are apt to occur, and they are generally accompanied by an extensively diseased condition of the lining membrane. These

coagula, from their laminated texture, and from the intimacy of their union with the internal lining, seem to be essentially different from polypi of the heart. They appear to be connected with a process of disease in the membrane, and to have obtained their union with it long before the death of the patient.

I once saw two separate tumors, entirely resembling what are called polypi, between the carneæ columnæ of the left ventricle, and firmly adherent to the heart; and in the centre of each a distinct formation of pus. The heart was otherwise healthy^[9].

There are three preparations in the museum of the College of Surgeons, put up by Mr. Hunter himself, and noted by him as exhibiting "Tumors on the inner surface of the right ventricle, seemingly composed of layers of coagulable lymph one upon another, the central part having the appearance of glary mucus." This last appearance (the mucus) no longer remains in any of the three, but in its stead there is a cavity. The tumors which I have mentioned as originally containing in their centre a distinct formation of pus, also now present in its stead a cavity. The preparation preserved at St. Bartholomew's, and those at the College of Surgeons, are probably specimens of the same disease.

Here what in the recent parts seemed to be pus or glary mucus, and the cavities still remaining in the preparations, must suggest the suspicion, that these tumors, whether they grew from the surface or were deposited from the blood, had become organized, and afterwards inflamed; and that the inflammation had terminated in the formation of an abscess.

ESSAY III.

Morbid Anatomy of the Muscular Substance of the Heart.

Dr. Baillie, speaking of inflammation of the substance of the heart, says, "When the pericardium covering its surface is inflamed, the inflammation sometimes passes a little way into the substance of the heart:" and then he gives these as the characteristic marks of such inflammation—that "it (the substance of the heart) becomes much more crowded with small vessels than in its natural state, and there are sometimes to be seen a few spots of extravasated blood." Now, this undue vascularity, and these few extravasated spots, where inflammation unquestionably belongs to a

contiguous structure, may be deemed sufficient evidence of the muscular substance of the heart participating, or beginning to participate, in the same disease; but, absolutely and exclusively, it may be doubted whether more is not required to give a certain assurance of its inflammation. Here, as elsewhere, we must look for some of the proper products of inflammation; or for some of the permanent changes of texture which naturally result from it, according to its several modes of disorganizing.

It is a very rare event to find pus among the products of inflammation of the substance of the heart: yet this event has occurred twice to my observation. In one instance the whole heart was deeply tinged with dark-coloured blood, and its substance softened; and here and there, upon the section of both ventricles, innumerable small points of pus oozed from among the muscular fibres. This was the result of a most rapid and acute inflammation, in which death took place after an illness of only two days. In another instance, after death, which terminated an illness of long duration and characterized by symptoms referable to the heart, a distinct abscess was found in the substance of the left ventricle, closed externally by a portion of adherent pericardium, and connected internally with an ossified portion of the lining membrane.

This diffusion of pus, or rather its formation in innumerable separate points, throughout the muscular structure of the heart, by a rapid and acute inflammation, is a singular occurrence. No such case has ever fallen within the knowledge of the most experienced in morbid anatomy, of whom I have made inquiry; neither has any such (as far as I can learn) been recorded in books^[10].

A single collection of pus^[11], constituting an ulcer or abscess of the heart, and resulting from chronic inflammation, is also a rare occurrence, yet not so rare but that a specimen of it may be found in most collections of morbid anatomy^[12].

But independent of the formation of pus, softening and attenuation, as well as induration and thickening of the muscular substance of the heart, do, in different cases, according to the circumstances under which they are found, both bespeak inflammation.

Together with the unequivocal evidence of inflammation in other parts of the heart, the muscular fibres have at the same time been found very soft

and loose, and easily torn; and with this looseness of texture the heart has sometimes presented a dark and almost black appearance, and sometimes it has been almost blanched and colourless. The deep dark tinge shews that the muscular substance is unnaturally loaded with blood; whereas the absence of colour shews that it is destitute of its natural quantity. These different appearances do, in fact, belong to different stages of the same disease. The first indicates inflammation of the muscular substance in its present state of activity; the second, an irreparable disorganization of the muscular substance left by inflammation, when it has been unarrested in its earliest stages. The first is found when death takes place *in a few days after* the accession of the disease; the second, when the patient survives the first attack, and dies at a remoter period.

The inflammation, which produces softening and attenuation of the muscular structure, is, I believe, always of an acute kind.

But induration and thickening of the muscular substance of the heart is also the result of inflammation. This must be distinguished from hypertrophy, or mere augmentation of bulk, of which we shall speak hereafter. In this induration, which proceeds from inflammation, there is, besides increase of bulk and firmness, a manifest alteration of texture. A substance^[13] is produced offering a peculiar resistance to the knife. This condition unquestionably must result from an interstitial deposition of new matter among the muscular fibres. From concomitant circumstances, as well as from its own character, I presume that it proceeds from chronic, not from acute inflammation. I confess that I never saw a specimen of what I now allude to. It is described with some minuteness by Corvisart; but it may be presumed to be very rare, since Laënnec admits that it never fell under his observation.

These opposite states of softening and attenuation in one case, and induration and thickening in another, are known to those conversant with morbid dissection as the evidences of inflammation in other parts of the body besides the muscular substance of the heart.

But there are changes of structure incident to the muscular substance of the heart which are independent upon inflammation, or, perhaps, upon any process which can properly be called morbid. They consist of simple augmentation and simple diminution of bulk and consistence. This simple augmentation of bulk and consistence is owing exclusively to a more ample

development of natural structure. The muscular substance is more red than natural, its carneæ columnæ are increased in thickness, and its proper fibrous texture is every where more strikingly manifest; but there is no interstitial deposition of matter new in its kind. It has been called hypertrophia of the heart, and the name conveys a tolerably just idea of its actual condition.

The simple diminution of bulk and consistence is a condition the exact opposite of the former. The muscular substance is less red than natural; its proper fibrous texture less distinguishable; but there is still the appearance of muscle shrunk and withered, as if from an insufficient supply of nourishment. It may be called atrophia of the heart.

Conjoined with augmentation, as well as with diminution of strength and bulk in the walls of the heart, there is almost always an increase in the capacity of its cavities; and in whichever part of the heart the walls are thus augmented or diminished, it is the cavity appertaining to the same part which undergoes the increase of capacity. Hence it appears either that one of these conditions is the immediate and necessary consequence of the other, or that both are the simultaneous effects of the same causes.

Dilatation of any cavity of the heart, with thickening of its walls, is called *active dilatation*; and dilatation of any cavity, with attenuation, is called *passive*^[14].

Active dilatation may appertain to every cavity of the heart simultaneously; and so may passive dilatation. But such occurrences are very rare: for one cavity being naturally more liable to this species of dilatation, and another to that, it most frequently happens that specimens of both conditions are found in the different cavities of the same heart.

The left ventricle is much more liable to active dilatation than the right; and the right ventricle more so than either of the auricles; and of the auricles unquestionably the left. Upon the whole, perhaps, the fact may be truly stated thus—that the left ventricle commonly gains an increase of bulk and strength, with an increase of capacity; and that all the other cavities, at the time that they expand, are rather apt to become attenuated.

It sometimes (though rarely) happens, that, with an increase of strength and bulk in its muscular structure, the left ventricle suffers a diminution of capacity in its cavity. In a case reported by Laënnec, the left ventricle was

an inch and a half thick at its broadest, and an inch thick at its thinnest part; and yet its cavity was only capable of containing an unblanched almond. I have seen the same condition in a less degree.—(*Laënnec*, vol. ii. 698.)

The circumstances under which these opposite conditions, this hypertrophy and atrophy of the heart, with the enlargement or diminution of its cavities, arise, must be explained hereafter. At present I would only remark that they are often found conjoined with actual disease in other structures of the heart, such as osseous or cartilaginous thickening of internal lining, adhesion of the pericardium, and ossification of the coronary arteries, to which they owe their origin; and, moreover, that they are often found where the heart is otherwise perfectly sound in texture, and where their cause must be sought in other parts of the body.

The heart is liable to undergo a simple dilatation of its cavities without either thickening or attenuation of its muscular substance. This simple dilatation is sometimes of the whole organ, sometimes of one side, and sometimes only of one auricle or ventricle. When it is of one cavity only, it may be complicated with active or passive dilatation of another, or of all the rest.

There is reason to believe that the heart sometimes undergoes a temporary dilatation, and again returns to its natural capacity; but that the dilatation can only subsist for a short time without becoming permanent.

The power of thus enlarging its cavities, and restoring them to their natural condition, belongs more especially to the right side of the heart.

A large accumulation of fat is sometimes met with about the heart.

The healthy heart is always more or less marked upon its external surface with streaks of white, and this appearance results from the deposition of fat in the cellular texture, which unites the serous covering with the subjacent muscular structure. It is found principally where the *venæ cavæ* unite to form the right auricle; also at the base of the ventricles, and along the line which marks the boundary between the two, and around the origin of the great blood-vessels as they emerge from the heart. But when fat is deposited in more than these situations, and in more than the natural quantity, it is not so much added to the healthy substance of the heart, as existing at its expense and detriment, and the muscular structure is that which especially

suffers. The fibre of the fat heart is pale and wasted, like that of a paralytic limb.—*Mus. of the Coll. of Surg.* 327.

A rupture of the heart is sometimes met with; but all the cases of reputed rupture are not such in reality. An aperture in the walls of the heart, through which blood escapes into the cavity of the pericardium, may result from ulceration as well as from rupture, or from a mixed process of one and the other; but whatever be the precise nature of the process by which this perforation of the heart is effected, it is undoubtedly of rare occurrence.

Where it has been found, there has generally been at the same time some peculiar condition of the organ, which might be presumed favourable to its production. Its muscular substance has been so soft and loose of texture, that it could be pierced through by the weight of a probe^[15]; or it has been converted into, or greatly intermixed with fat^[16]; or its muscular fibres have been absolutely defective, leaving a certain transparent space, where the internal lining and the pericardium have been in contact, and served to maintain the completeness of the cavity, until the rupture has taken place^[17].

Not that a rupture has not been found where the texture of the parts has seemed to offer no natural facilities to its occurrence—as in the case related by Harvey himself, who found a lacerated aperture in the left ventricle, capable of admitting his finger, through which blood had escaped into the pericardium, the walls of the ventricle being increased in thickness and strength, while an obstacle existed at the entrance of the aorta^[18]. Here the heart must have torn itself asunder by the simple violence of its contraction, in contending against the impediment to the egress of blood from its cavity. This is an effect which would hardly be thought capable of being thus produced; but I can well believe it possible after having seen one of the recti muscles of the abdomen literally torn in twain, in a man who died of tetanus^[19].

[To be continued.]

DILATED ŒSOPHAGUS.

To the Editor of the London Medical Gazette.

SIR,

Permit me to send the following case to you for insertion in the Gazette.

Mary Blores, æt. 33, was admitted into the Middlesex Hospital on the 16th of November. She was in a state of extreme feebleness and emaciation. Those who brought her said that during the preceding month she had appeared to swallow nothing: what she took as food seemed to her to stop in the gullet, and after a few minutes returned. I found, however, that a large œsophagus-bougie passed readily into the stomach, meeting with no obstruction beyond a spasmodic resistance in the pharynx. A draught of milk and water was given her—she swallowed it without much effort, but it quickly returned. In the course of the afternoon a pint of beef tea was injected through an elastic tube into the stomach—it was thrown up directly.

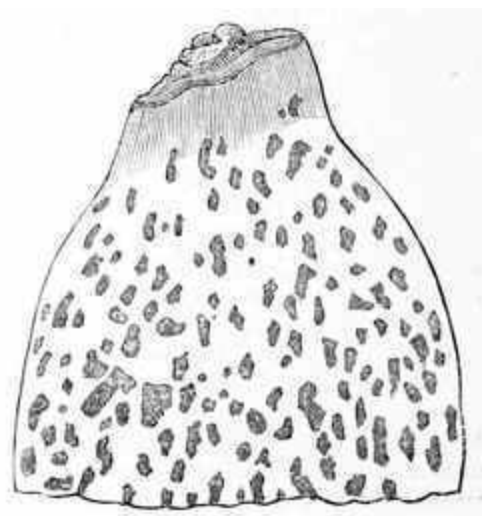
I now learnt from her that she could swallow liquids much more readily than solid food; that when she took a small quantity, it seemed to her not to reach the stomach, and in three or four minutes was invariably thrown up; that on taking a large draught she had an impression that it reached the stomach—in this case vomiting did not follow so soon, and some part of the draught was permanently retained; that she craved food and drink, and was literally dying of hunger and thirst; that the vomiting which took place was not preceded by nausea, although in its progress it had all the appearance of ordinary retching; that the matter vomited was not thrown up at once, but by successive efforts; it consisted of the food she had last taken, mixed with colourless mucus. The belly was so shrunk that the umbilicus was not more than an inch distant from the spine, upon which the pulsations of the aorta were readily felt: there was no enlargement or hardness about the stomach, no particular tenderness on pressing the epigastrium, no sensation of pain or heat now or formerly.

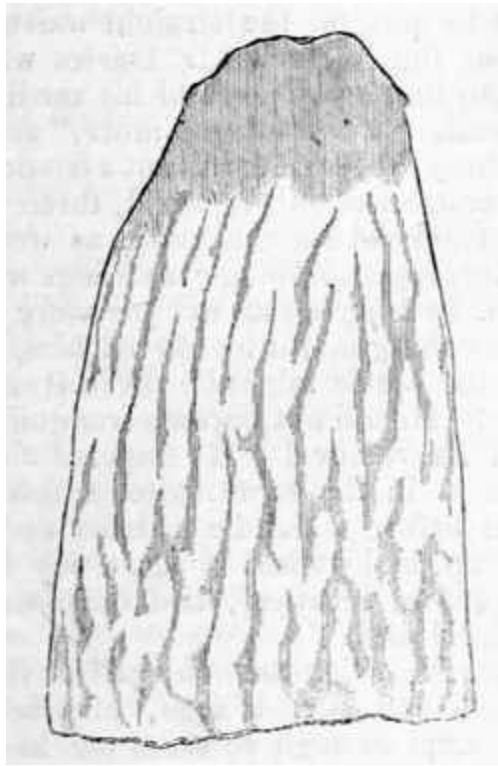
The complaint had began ten years ago, during pregnancy, since when she had never been free from it, although at times her sufferings had been

less, and she had been able to retain some portion of her meals. She had borne in this period three children; the vomiting had lasted during the whole period of her pregnancies, and during her confinement. The principal remedies which had been used for her relief were a succession of blisters on the pit of the stomach—but they had produced no benefit. Latterly her symptoms had been greatly aggravated.

This patient was attended by Dr. Watson and myself. But the means which we tried to enable her to retain her food, and to support her expiring strength, had scarcely a temporary effect. She died on the 2d Dec. sixteen days after her admission. The body was carefully examined, at the expressed wish of her relatives.

The unusual appearances found in the abdomen were—1. the smallness of the first part of the duodenum, which was but half the ordinary size of the ileum; 2. the capaciousness and fulness of the gall bladder,—from which, however, on compressing it, the bile flowed readily into the intestine; 3. a contraction of the middle of the stomach of the length of two inches, for which extent the peritoneal coat was thickened and opaque, and the inner membranes folded in deep longitudinal rugæ, the mucous surface of which was partially suffused with circular spots of red. The breadth of the contracted part of the stomach, as it lay collapsed, was an inch and a half.





But it was in the chest that the most remarkable circumstance presented itself. The œsophagus gradually enlarged from the pharynx, which was perhaps rather narrower than usual, to an extraordinary degree of dilatation; the greatest breadth which it attained was situated about four inches above the cardia: the tube then contracted more abruptly, so as to render the termination of the œsophagus, like its commencement, of nearly the usual dimensions. The structure likewise of the cardiac extremity for about an inch, and of the pharyngeal end for about half an inch, was healthy. The intermediate part presented, when inverted, the following curious appearance:—The inner membrane was thickened and opaque, and had *the appearance* of having partially yielded from dilatation; at the upper part the furrows or thinner parts of the membrane followed in some degree a longitudinal direction; at the lower part the surface was pitted with shallow depressions of various figures. I have represented in the adjoined sketches these appearances, which on the middle of the œsophagus passed from one into the other; at the furrows or depressions the membrane seemed of the natural thickness and colour; the intermediate raised and thickened part was

opaque and whitish. The muscular fibres of the œsophagus were of the natural colour and thickness.

I remain, Mr. Editor,

Your obedient servant,

HERBERT MAYO.

19, George-street, Hanover-square,
Dec. 6, 1828.

HYDROPHOBIA.

To the Editor of the London Medical Gazette.

SIR,

As the following case may throw some additional light on the pathology of hydrophobia, I have sent it you for publication.

I am, Sir,

Your obedient servant,

F. GODRICH.

Grove House, Little Chelsea,
Nov. 29, 1828.

On Thursday morning, the 25th ult. I was called up about seven o'clock to see a man who I understood was exceedingly ill, and waiting in the surgery very impatiently for my arrival. I found my patient (Mr. Barham), a fine looking old man, about 60, labouring at intervals of about five minutes under strong spasmodic paroxysms, affecting the muscles concerned in breathing and deglutition. There was a wildness and an impatience depicted in his countenance, totally different from any thing I had ever observed in other spasmodic affections. His bowels were open, tongue clean, skin moist, pulse full and a little accelerated. I took away twenty ounces of blood, and prescribed a mixture containing ʒss. of laudanum for a dose every hour until I should see him again. He walked home half a mile from my house (Gloucester Road, Old Brompton), and left me ruminating on the possible cause of so much mischief occurring suddenly in a fine healthy subject. At ten a message was sent, saying that he was much worse, and requesting me to call as soon as possible. He received me tranquilly, and said he was very glad that I had come to see him, for he was very ill. His symptoms were now more distressing than when I first saw him: he looked wildly and suspiciously at every one entering his apartment, and his breathing was accompanied by a short convulsive sobbing. On looking at his medicine I perceived he had taken none, and expressing my surprise, he assured me it was impossible for him to swallow a single drop, as the attempt had been followed by violent spasms, and produced so much distress, that he had

desisted. At this period no one had the slightest idea of the origin of his malady. I poured out some medicine into a tea-cup, the very act of which produced much excitement and alarm. My first impression as to the true nature of his disease arose at this period, from the circumstance of his requiring a tea-spoon, with which he endeavoured to take some of the medicine. The attempt produced much excitement and alarm, and after two or three painful efforts at deglutition, with one desperate effort he swallowed a tea-spoonful, threw away the spoon, and begged, unless I wished to destroy him, that he might have nothing more to swallow. I now left his room, and inquired of a bystander whether any thing particular had occurred to him within the last few weeks. On recollection she said, "About a month since, late at night, a strange dog came into the premises and fought with his own dog; he got out of bed to separate them, and the strange dog bit him in two places, on the left arm and hand; and bit a puppy, which died about a fortnight after in a strange way, which was thought to be some kind of a fit." To ascertain if this occurrence had produced any effect on his mind, while again bleeding him I said, "You have been in the wars, Sir, and had your hand and arm torn; how did it occur?"—"Oh!" said he, carelessly, "that was done by a dog a long time ago, but it healed." The circumstance was never again mentioned to him, and he died in total ignorance of the cause of his malady. The wounds were perfectly cicatrised, and there was not the least action going on indicative of recent absorption. He bore the bleeding pretty quietly: 40℥. were removed, which on cooling presented strong marks of inflammation.

Ordered ℞iv. Acid. Hydrocyan. omni horâ, in a little water.

Twelve o'clock.—With much difficulty he has taken two doses of the acid; pulse full and hard, 110. 30℥. more blood were removed.

Three o'clock.—Has taken two more doses; complains of a dreadful sense of suffocation, and implores that nothing more may be given him. Pulse full, and beating at 120 to 130. Continue the acid.

Eight o'clock.—Pulse full and hard. Has taken in all 24℥. of the acid, but so painfully distressing has the deglutition now become, that all attempts at repeating his medicine are discontinued.

V. S. ad ℥xxx.

During the bleeding he looked wildly at the basin, and begged that no more might be spilt (a drop or two had fallen), repeating frequently, in great agitation, as the blood was running, "Take care! take care!"

Between two and three o'clock next morning my assistant (Mr. Davies) visited him. He found him tolerably passive, but observing every movement with intense anxiety. Pulse full and hard, face flushed, eyes denoting cerebral irritation. He had been at times outrageous. On its being intimated that bleeding was again necessary, a paroxysm came on more intense than any preceding,—and with great effort he submitted. As the blood flowed he became more and more alarmed, till at length he got quite unmanageable; he raged violently at his nephew, who was holding the basin, and ordered it peremptorily to be removed. 30 or 40℥. were taken away. It was found necessary to put on the straight waistcoat. About four o'clock Mr. Davies wished him to take some more of his medicine. He said, "I can take no more," and on reaching the bottle to put out a few drops, he became violently agitated, threw himself from side to side, and, as well as the incessant spasmodic sobbings would allow, he begged that not one more drop of any thing might be offered him, and that the bottle might be taken from his sight. He did not become tranquillized until its removal. He lingered on till ten A.M. in the same state, a few minutes before which he insisted on getting up, and walked a short way down his garden, returned, laid down on his bed, and died.

Mr. Frederick Salmon, of Old Broad Street, and Mr. Wilson, of Chelsea, were kind enough to assist me in conducting the post mortem examination. On opening the chest, the heart was free from disease, with rather more water in the pericardium than natural; the lungs were completely gorged with grumous blood, and the pleura adherent on the right side. On removing the cranium, which was remarkably thin, and cutting the substance of the brain, numerous red spots presented themselves in the medullary portion; about a table-spoonful of water in each ventricle; the plexus choroides was turgid; the corpora, striata, thalami, and basis of the brain every where preternaturally injected; the cerebellum, crura cerebri, and cerebelli, in a high state of inflammation. On removing the spinous process of the vertebra, the whole cord was considerably inflamed; and opposite the two last cervical and dorsal vertebræ the cellular substance was studded with dark patches of coagulated blood, the theca vertebralis thickened, and the

cord in an active state of inflammation. The larynx and pharynx bore not the slightest vestige of disease. The preparation of the cord is deposited in the museum of the London University.

The post mortem examination of this case tends to prove the correctness of Professor Thompson's theory of the proximate cause and seat of this afflicting malady; and the plate accompanying a case recorded by him, in the 13th volume of the Med. Chir. Society, gives a faithful delineation of the state in which the spinal cord was found in this case.

CASE OF LONG PROTRACTED CONSTIPATION.

To the Editors of the London Medical Gazette.

GENTLEMEN,

Perhaps the following case may interest some of your readers, and if you think so, its insertion in your Journal will oblige

Your obedient and
humble servant,
S. D. BROUGHTON.

12, Great Marlborough Street,
Dec. 3, 1828.

On Tuesday, Nov. 25, I was called upon at the St. George's and St. James's Dispensary to advise for an infant at the breast, of five months and fourteen days of age, that had not passed any fæces during six weeks up to the day before I saw it. After its birth it had no passage till the fifth day, and this had apparently been effected by the introduction of a bougie, and which was subsequently continued at intervals for a short time. The infant was otherwise healthy and strong at its birth, and remained so until it had gone about three weeks without any evacuation, when it was observed to be weak in the lower extremities; and at the period of my seeing it the legs were much fallen away in flesh, hanging flaccid and inert. Many aperient enemas and medicines had been given, and, as a last resource, one drop of the croton oil had been prescribed the preceding evening. The infant appeared to suffer from distention, and to be disposed to coma, and the croton oil had distressed it much some hours after it was given. The fæces could be felt blocking up the intestine, but the irritation of the finger produced no attempt at evacuation; very small pieces only having been brought away. Hot baths also had frequently been used some time ago.

Considering the case as one of paralysis in the parts naturally employed to relieve the bowels, I recommended the suspension of all purgative remedies by the mouth, and a table-spoonful of the following injection to be substituted, and to be thrown up with three table-spoonfuls of thin gruel occasionally during the day, by means of a syringe.

Rx Spir. Terebenth. ʒiij.
Træ Zinzib. ʒj.
Aquaë, ʒiss. M.

The following liniment was also directed to be rubbed in frequently along the course of the spine.

Rx Linimenti Saponis, ʒiss.
Liquor. Ammon. Comp. ʒss. M.

The result justified my expectations; for, during the night after this prescription, two injections having been given, some large and hard ash-coloured lumps came away, and these were shortly followed by a forcible spouting out of a continued stream of feculent dark liquid; a brisk diarrhœa having been kept up till the Saturday following. The infant's appetite is become voracious, its appearance lively and healthy, and its legs as firm and strong as is usual at such an age. Cold water is recommended to be dashed upon the loins, the liniment to be continued occasionally, moderate doses of magnesia and rhubarb to be given when required, and the turpentine injection to be used if any return of the complaint should be threatened.

DISLOCATION OF THE PATELLA.

To the Editor of the London Medical Gazette.

SIR,

Having seen a case of dislocation of the patella, reported by Mr. Mayo, in your Journal a few weeks since, in which there was experienced some difficulty in the reduction of the bone, I beg leave to send you the following, in case you may think it worth insertion.

On Saturday last, I was called to a young woman who had dislocated the patella of the left knee joint outwards; the inner edge rested upon the outer surface of the external condyle, and was immoveably fixed, and caused great pain when touched. She says it was occasioned by her foot slipping backwards from one stair to another, and she immediately fell down.

I commenced the reduction, as stated by Mr. Mayo, but I could only produce semiflexion, and that gave her great pain; insomuch that I was obliged to desist, and the patella remained in *statu quo*. But when the knee was thus semiflexed, I desired my assistant to extend the leg suddenly; and having my fingers on the patella at the time, and pressing it inwards, it immediately returned to its natural position.

I must confess, sir, that I should have been afraid to have continued the flexion as described in Mr. Mayo's case, "by bending the knee to the utmost," fearing that the remedy might be worse than the disease, and the result proved that in this case it was not necessary.

I remain, Sir,

Your obedient servant,

P. M. Hoskings.

168, Fleet-Street,
Dec. 3d, 1828.

VACCINATION.

To the Editor of the London Medical Gazette.

SIR,

The mistake of the printer in substituting the word "every" for "even," in the paper on Vaccination (p. 40), so materially affects the sense of the passage, that I must request the favour of the following insertion:—

"3d. That while the vaccine cicatrix retains even one distinct fovea or depression, the individual continues unsusceptible both of small pox and of re-vaccination; but that," &c.

I am, Sir,

Your obedient servant,

THOS. H. BURDER.

2, Brunswick Square, Dec. 15, 1828.

SINGULAR BICEPS MUSCLE.

To the Editor of the London Medical Gazette.

SIR,

The valuable paper communicated to your Journal, by Mr. Stanley, last week, has brought to my recollection a singular appearance of the parts in front of the shoulder-joint which I dissected about two years ago. It is a fact worth recording; and perhaps you will be kind enough to insert it in your next number.

The circumstance was this—that in the right arm, instead of there being a biceps muscle having two origins, the one the shorter from the corocoid process, the other the longer from the upper part of the glenoid cavity of the scapula, there was simply a one-headed muscle arising from the corocoid process. I could find no tendon passing through the joint answering to the long head of the biceps, neither was there any appearance of a biceptical groove; the capsular ligament was, perhaps, thicker than it is found in ordinary cases.

I knew nothing of this man during life, and am therefore unable to give any account of the use which he made of this arm.

I am, your obedient servant,

C. M. BURNETT.

House Surgeon's Apartments,
St. Bartholomew's Hospital, Dec. 8th.

STETHOSCOPE.

To the Editors of the London Medical Gazette.

GENTLEMEN,

As you did me the honour, on a former occasion, to publish some remarks in defence of *auscultation*, against its enemies and detractors, I trust you will permit me to say a few words in reference to a communication in a late number, from a gentleman who appears to be a friend of that method of diagnosis, but whose ignorance of the subject is likely to be more injurious to the cause than either open enmity or secret detraction. The case in question (page 780, vol. ii.) appears, from the dissection, to have been clearly *emphysema of the lungs*, as there stated; but I appeal to every one practically acquainted with the physical signs of diseases of the lungs, whether the results of the exploration with the stethoscope, as narrated in the case, were those pathognomonic of that disease? I say they were not only not pathognomonic of this affection, but they were actually incompatible with its existence. The same ignorance of the subject is shewn in relation to the disease of the heart, which, I boldly maintain, no one was justified in pronouncing to be *hypertrophy of the left ventricle* from the stethoscopic indications stated in the case.

It is just possible that the writer of the case may have made further explorations on which his diagnosis was founded; but if he had such in his case-book, it is equally unfortunate for the credit of the stethoscope and his own, that he suppressed them in his printed case.

I have the honour to be,

Gentlemen,

Your obedient servant,

A STETHOSCOPE.

Nov. 25, 1828.

MECHANISM OF VOMITING.

To the Editor of the London Medical Gazette.

Glasgow University, Nov. 26, 1828.

SIR,

Having lately perused, in the 35th Number of the Medical Gazette, some observations "on the mechanism of the act of vomiting," by Dr. Marshall Hall, in which, after controverting the different opinions which have been entertained by physiologists on this question, he endeavours to prove that vomiting is an effort, not of *inspiration*, but of *expiration*, and advances this opinion as having originated in himself, I beg, through the medium of your Journal, to refer that gentleman to Richerand's Physiology, where, on the subject of vomiting, page 70, he says, "Sometimes the action of the muscular fibres of the stomach is quite inverted; they contract from the pylorus towards the cardia; and this antiperistaltic motion, in which the contractions act with more force and rapidity, and in a manner decidedly convulsive, produces vomiting. The action of the abdominal muscles then connects itself with that of the stomach; the viscera are propelled upwards and backwards, by the contraction of the large muscles of the abdomen; *the diaphragm ascends towards the chest; if it descended, in contracting, the æsophagus that passes between its two crura would be compressed, and the expulsion of food through the cardia could not be effected.* It is also observed, *that during expiration only, any thing can pass from the stomach into the æsophagus.*" Again, on respiration, page 147—"In every effort of *expiration*, as coughing and *vomiting*," &c. Here, then, we see that Dr. Hall's views, as far as regards vomiting being an effort of expiration, have long since been advanced by Richerand.

The profession are certainly much indebted to Dr. Hall for his corroboration of these views; at the same time he should not have taken to himself the merit of having first suggested them.

Sir,

Your most obedient servant,

A SUBSCRIBER.

CHARACTER OF THE LANCET.

To the Editor of the London Medical Gazette.

Dec. 19, 1828.

SIR,

The events disclosed during the late trial in the Court of King's Bench, must fill every well-disposed mind with astonishment and horror. The moral aspect of the case is marked by every feature capable of exciting a sense of shame for our profession, and of sorrow for our kind. Can it be that a publication conducted in the spirit of the extracts produced at the late trial of Cooper against Wakley should be fostered and encouraged by a large proportion of the medical public? Is it indeed true that no feeling of loathing and disgust should have induced the purchasers of the *Lancet* to protest against its glaring injustice, its undisguised ferocity and malignity?

I am not a reader of that work myself, and but for the disclosures recently made in our courts of justice, together with what I have learned from your own pages, I should know little about it. Some of its early numbers convinced me that it was a production not deserving of support, whatever ability there might be connected with it; subsequent events have amply justified this decision, and I now address these few hasty lines to you, to express my humble hope that all the respectable medical men throughout the kingdom will arise, and with one voice oppose this moral pestilence, which at once corrupts and degrades their profession.

The *odium medicum*, bitter and vindictive as it unfortunately has been, never, I believe, appeared in a form so repulsive. Are we living in the nineteenth century? Have arts and sciences, which are asserted of old to soften the manners and temper the violence of our nature, produced no such fruits amongst us? Is serious, and perhaps irremediable injury, to be inflicted on reputation; and is the sufferer thereafter to be dared to mortal combat, and this too by the wrong-doer? Is it thus that professional justice is to be awarded? Is it thus that knowledge is to be advanced, the dignity of our profession to be maintained, its benefits enlarged, and its usefulness transmitted? Is it thus that the young men are to be taught how to acquire an unworthy and discreditable notoriety; to disregard all the decencies, charities, and higher virtues of life; and to bring into the exercise of our art

the insidious cunning of the knave, the vulgar dishonesty of the pilferer, and the audacious bearing of the bravo?

I know not how others may feel on the present occasion: I am entirely unacquainted with the parties who have been at issue in the late suit—I have no bias of any kind, but what must arise in the mind of every man who has any regard for the profession to which he belongs, or who loves truth and fair dealing—I am sure that it is not less for the interests of society at large than for the advancement of medical science, that just principles should regulate all our dealings, both with the public and with each other. It is manifest that of late we have signally and woefully departed from them; and it is most distressing to know that this departure has been sanctioned by so many members of the profession. I now entertain a confident hope that all who reflect on the character of the late libel will feel that their own *character* is at stake if they do not mark their disapprobation of it in every possible way. Sure I am that, if the principles which guide the Lancet become generally acceptable, it will mark an æra of moral degradation which no surgical dexterity, no medical acquirements, however eminent, could possibly redeem.

VERITAS.

DURATION OF HUMAN LIFE IN RUSSIA.

Cases of longevity are not only much more common, but also more extraordinary in respect to a greater duration, in Russia than in any other part of Europe; thus, from the report of the holy synod, published in 1827, it appears that there were living in 1825, among those who professed the Greco-Russian religion throughout the empire, not fewer than 848 males who were 100 and more years old; among whom, 32 had passed the age of 120, 4 were between 125 and 130, and 4 others between 130 and 135 years of age. Out of 606,881 males who died in 1826, 2785 had passed the age of 90 years; 1432 that of 95; and 818 that of 100. Among the latter, 38 were more than 115 years of age; 24 more than 120; 7 more than 125; and one was 160 years old at his death.

MEDICAL GAZETTE.

Saturday, December 27, 1828.

"Licet omnibus, licet etiam mihi, dignitatem *Artis Medicæ* tueri; potestas modo veniendi in publicum sit, dicendi periculum non recuso."—CICERO.

ANALYSIS OF WAKLEY'S DEFENCE ON THE LATE TRIAL.

We last week presented our readers with a full^[20] and, we trust, accurate account of the trial for libel, which has recently excited so much interest; and we offered some remarks of a general nature upon its result and tendency. We had purposed at the same time to subject Mr. Wakley's mode of conducting his cause to a critical investigation, but this we were prevented from doing by want of time and space. We thought it likely, too, that the Editor of the *Lancet* would have fairly met the question in his next publication, and himself put the facts of the case, at least, on record: instead of this, however, he has devoted his pages to compliments to himself, and scurrilous and absurd abuse of Sir James Scarlett, in which he informs us that in his opinion that gentleman is "prodigiously overrated even as a lawyer," and says many other things, which serve only to shew how dreadfully Wakley himself had smarted under his lash.

We find the general opinion to correspond with that which we expressed last week—that the defendant had managed his cause in a very bungling manner; and the estimation in which he and his witnesses were held by Judge and Jury is apparent from the charge of the one, and decision of the other. Both, indeed, regarded the asseverations of his whole *family party* as dust in the balance against the testimony of one single competent and honourable man—Mr. Callaway; for on his evidence the result obviously hinged. Our contemporary affects to look upon the verdict as a triumph on his part—so he would if the damages had been five hundred or a thousand pounds, for in truth he has nothing else for it; and with all our hearts we wish him many such victories.

With regard to the amount of damages, we certainly should have been better pleased had it been larger; but we did not expect that it would—nay a reference to our number which was published while the trial was going on, will shew that we were even doubtful of the verdict, because we know how extremely difficult it is for a jury to comprehend such a complicated question—how easy for a knave to impose upon their ignorance by misrepresentation and falsehood. It is quite obvious that if the jury had believed one syllable of the evidence of Wakley's witnesses, they must have given the verdict in his favour. In a case like this, where *one individual only*, and he the plaintiff, can be fully aware of the difficulties which had to be overcome, it is almost impossible to bring evidence into a court of justice capable of satisfying the minds of twelve unprofessional men. From this circumstance alone it was that we expressed a doubtful opinion of the result. But besides all this, it is well known to all—that *one talesman might go determined to starve out the rest—that ten special jurymen might wish to give 500l. and yet that, by the absurd constitution of the English law, their verdict might be thus controlled by one individual friendly to the defendant.*

Most fortunately for the cause of truth, Wakley had the vanity and folly to conduct the defence himself, and, judging of other men's estimation of moral character by his own, he did not hesitate to place in the box witnesses whom no counsel would have suffered to go there, and who, being unable to stand the cross-examination, led to what must have been to Wakley a most humiliating exposure. Next to this, the most beneficial effect produced was that of the defendant's cross-examination of his opponent's witnesses: it was amusingly absurd, and shewed a lamentable want of tact, always eliciting something which he ought to have avoided. Lastly came the reply, of which he tells us he is himself ashamed—"We may state most unfeignedly," says he, "that we look back with regret to our errors in point of omission." Nor is this the only part of it for which he has cause to blush, for any thing more miserably deficient we never heard: it was a mass of confusion throughout—a repetition of the same ideas and expressions—and a most injudicious betrayal of rancorous feeling, at the very moment he was profanely invoking heaven to witness that he bore enmity to no man. In speaking of his reply in our last number, we declared it to be "false in statement—sophistical in argument—coarse in language—and diabolical in tendency;" and we now proceed to illustrate these several qualifications, in

doing which little more will be necessary than to select a few passages, and arrange them under their appropriate heads.

False in Statement.—One of Wakley's principal objects was to impress upon the Jury a belief that he had not in any way attempted to influence his witnesses, nor even spoken to most of them, anterior to the trial. "Out of nine witnesses I have placed in the box, (says he) I never saw five of them, nor even spoke to five of them till yesterday." We appeal to any man of common sense, whether it be probable that this is true. It is possible indeed, and only just possible, that he was contented with employing others to question his witnesses, and ascertain what evidence they would give; but even if it were so, the assertion was not the less false in a moral point of view, because his intention was to convey the belief that he had inquired no farther than merely whether or not the witnesses had been present at the operation. But incredible and absurd as this was, he ventured a step farther, and assigned a reason for the choice made of one witness—(Lee the potatoe-merchant.) "I had heard (said Wakley) he was present at the operation—an honest man, and that he was a friend of Mr. Bransby Cooper, under the highest obligation to the Cooper family. Without asking him a single question I put him into the box, as I was most anxious you should hear all the evidence that could be adduced, and from *unsuspected and untainted quarters*." This lie carried with it its own refutation; for Lee swore that he had never even spoken to Mr. Cooper; and we are therefore amazed that Wakley should have hazarded so gross an assertion, as it could only have been effective on the supposition of his addressing a jury of absolute simpletons. But again—"Who were my witnesses? not men whom I had trained and lectured on models, and sketches, and drawings. Has it been shewn to you that I was closeted with any person or set of persons?" Yes, Mr. Thomas Wakley, it has been shewn, and by no less an authority than that of your *fidus Achates*, Mr. James Lambert. "The defendant (says the worthy author of the libel), the defendant was there whilst I was explaining the parts to persons who were to be witnesses—the defendant also explained them to the same persons." And again, "I have heard explanations given to persons who were to be witnesses in this cause FOUR OR FIVE TIMES within the last six weeks." So also that excellent thorough-going witness, Mr. Jeffry Pearl: "a gentleman of the name of Lambert introduced me to the defendant; THE DEFENDANT and Mr. Lambert endeavoured to shew that the forceps had passed between the bladder and

the rectum." So much for placing men in the box because they are friends of Mr. Cooper—so much for procuring evidence from "untainted quarters!"

This is only to be matched in effrontery by the statement of some wise-acre at the radical meeting on Tuesday night. Being asked why no notice had been taken in the *Lancet* of a successful operation of lithotomy performed by Mr. B. Cooper, soon after the publication of the libel, he assigned as the reason that Mr. Wakley thought it might increase the damages, and otherwise injure Mr. Cooper! What a noodle the man must be to expect any one to believe him^[21]!

Mr. Wakley asserted that the evidence of his witnesses remained "untouched—wholly uncontradicted;"—whereas there was scarcely a single point in their statements which was not contradicted either by Mr. Callaway, who was present at the operation, or Mr. Key and Dr. Hodgkin, who examined the parts after death. But what is more remarkable, there was scarcely a circumstance on which his own witnesses agreed—for example, they differed with regard to the nature and direction of the incisions; with regard to the manner and frequency of introducing the forceps; with regard to the number of instruments used, and the mode of applying them; with regard to the size of the wound; with regard to the degree of force employed, and with regard to the appearances after death. Yet, in the face of all this, Wakley had the impudence to tell the jury that his evidence remained "uncontradicted." We have no hesitation in asserting that his own witnesses (putting Mr. Cooper's out of the question) advanced no charge of which they did not also furnish the refutation.

Mr. Wakley asserted that Mr. Cooper was incapable of telling why he could not extract the stone, even "while he had his finger in the bladder." This is false: neither Mr. Cooper nor Mr. Callaway were able to reach the bladder with the finger at all, owing to the depth of the perineum.

Mr. Wakley told the jury that the plaintiff did not like the bladder to be exposed, "because to surgeons there is indisputable proof that Bransby Cooper did not perform that operation as he ought," and that "Mr. Key and Mr. Callaway knew it." This is false: Mr. Key, Mr. Callaway, and Dr. Hodgkin, who had examined the parts, all deposed that there was nothing in them indicative of the operation having been improperly performed. Indeed, had it been as Wakley asserted, the plaintiff would not have been so foolish as to have had the parts preserved at all.

Sophistical in argument.—One of the pieces of sophistry on which Wakley seems chiefly to have depended, inasmuch as he went over it *six times* in the course of his speech, was, that if his account of the operation had been incorrect more of those who were present would have been brought forward to prove it so. But, as we stated in our last Number, to have objected to Wakley bringing forward pupils, on the ground of their being incompetent, and then to have done the same thing, would have been grossly inconsistent. Lord Tenterden, in his charge, said, "he did not see whom he (Mr. Cooper) could, with propriety, have called besides Mr. Callaway; the others were all young men, only pupils, and probably too young and inexperienced to form any judgment on the case that could be relied on." There is also another person who has objected to these witnesses whom we may mention—not, indeed, one for whose authority we have the highest possible respect, but of whom the defendant in this case probably thinks more favourably—we mean Mr. Thomas Wakley, who, speaking of the Letter formerly published by Mr. Cooper's pupils on the subject of this operation, says, "their united opinion of the skill, dexterity, and self-possession exhibited by Mr. Bransby Cooper on this occasion, is not likely to influence the judgment of the profession, whatever it may effect with the public."

The use made by Wakley of Sir Astley Cooper's expression, "give him time," is another specimen of sophistry. Sir Astley evidently meant merely to say that, *ceteris paribus*, a man who had been twenty years surgeon to an Hospital would be more experienced than one who had held the situation but one-fourth of that period. This we believe to have been his sentiment, though nothing was ever less happily expressed. Accordingly it was not lost upon the defendant, who exclaimed, "Is that the way in which our hospitals are to be conducted? Is that the mode in which your poor patients are to be treated?—that young and inexperienced men are to be placed there to *learn their profession*—not to know it before they get there, but are to go there to learn it—and learn it upon whom?—upon individuals who are as much entitled to the best and most scientific practice of surgery as any nobleman in this land." All very fine—but we should be glad to know how or where a practical acquaintance with surgery is to be acquired but in public hospitals—and if no man is to be appointed till after he has gained experience, where are such men to come from? Where are ready-made surgeons to be found fit

to operate even before Lambert, without his being able *to make a hole* in their reputation?

One of the most knavish attempts in the whole speech, to take advantage of the jury, was exhibited in his description of the operation. How different from the clear, perspicuous, and candid narrative of Sir James Scarlett. You are told, says Wakley, that the stone was shelved *above* the pubes and *behind* the pubes; but, continued he, (placing a pelvis in the position in which it is during the operation,) a stone cannot be lodged above the pubes, because, you see, the pubes is above the bladder! This was, indeed, calculated to confuse men unaccustomed to the subject, and who did not probably reflect that above and below are merely relative terms, which must change with every change of position, and that it was not meant that the stone was between the pubes and external integuments, which, in the position for lithotomy, would be the literal meaning of *above the pubes*, but that it was in such a situation as, in the ordinary position of the body, was above the pubes. But Wakley assured the jury that it was impossible for any stone to be *above* the pubes, for there was a tendon to prevent it which no force could overcome; and he affected to ridicule the idea of there being anything unusual, any extraordinary shelf behind the pubes, in which the stone could have been lodged. "Had there been a shelf here," said he,— "had there been a crooked passage—had there been a cavity, or any place for the deposition of the stone, do you suppose that shelf would not have been produced? Had there been any thing particular in the formation of the pubes, would they not have cut out the man's bone as well as the bladder?" It is probably impossible for any but medical men to appreciate fully the impudent imposition—the fraud of this statement,—this knavish artifice to mislead the jury. No attempt was made in any part of Mr. Cooper's case to show that there was such malformation, or that the stone was in the position which Mr. Wakley demonstrated it could not occupy—*thus answering an argument which had never been advanced*.

The following is a good specimen of sophistry and stupidity. Finding it proved that there was no opening between the bladder and rectum, Wakley wanted to make out that he never had said otherwise, having stated the cellular membrane to be *lacerable*, and therefore not *lacerated*. And then, forgetting the nature of his argument, immediately added, "remember this; the gorget is not as wide as the finger, and that it is not an uncommon thing

for an unskilful operator to drive the gorget in between the bladder and rectum." Thus insinuating what had been proved to be false, and what he had just argued, from the words of the libel, he could not have meant.

Coarse in language.—This scarcely requires illustration. But as specimens, on the present occasion, of vulgar slang and miserable attempts at wit, we may mention his talking of "hole and corner" proceedings,—his comparing Sir James Scarlett to sour milk, and which happy thought he has, in hand-bills pasted about the town, converted into "Knight and Professor of Sour Milk,"—his explanation of the term *bat* as applied to surgeons, "because they live in the dark, they suckle their young, and live in creaks and crevices of old walls, hospitals, and dungeons, *thriving and fattening on their own species*,"—his address to the "*gang*" of hospital surgeons, whom he represented as wading "through blood up to the neck,"—and lastly, his exclamations, several times repeated, perhaps with *prophetic* phrenzy, that he "would die in a dungeon and expire in a ditch."

Diabolical in tendency.—The general tendency of Wakley's writings, and especially of his address to the Jury on this occasion, so far as such ravings can have any influence, is to sow distrust between men in every class of society and their medical attendants; to poison the sources of public charity, and arrest the hand of benevolence; to contract the relief afforded to human suffering, leading the poor and uninformed to suppose themselves the objects of cruel and wanton experiment, thus encouraging them rather to suffer disease to run its course than seek relief in such abodes of blood and horror as he most falsely represents our public hospitals to be.

One of the first ebullitions of this demoniacal effort consisted in representing to the Jury, which he did in two different parts of his address, that the patient had been kept *bound* during the operation, as if some unusual degree of coercion had been employed. "Yet, in opposition to the patient's cries—in opposition to his repeated entreaties to be unbound, still the operator kept him upon the table."—"Gentlemen," continued he, "I should like to know upon what principle it was that the man was kept *bound* upon the table when he implored to be loosened. Was he not a free agent?" And then he goes on to put into Mr. Cooper's mouth not only words, but a sentiment he never uttered.—"No, no, (says the operator,) my *reputation* is concerned. You were brought here to have the stone extracted, and extracted it shall and must be—if you were to die upon the table." Unless Wakley was

intoxicated we cannot in any other manner account for an argument at once so absurd, so unprofessional, and so malignant. It evidently excited no feeling but that of disgust in the minds either of the Judge or the Jury.

The next paragraph to which we shall allude is a masterpiece of hypocrisy—"Let me ask you (said he) what you have heard respecting the miseries of the individual on whom the operation was performed? Not one word has escaped the lips of the counsel on that subject—not one word from the mouth of one of the witnesses. No, gentlemen, they are hospital surgeons themselves, and they know too well what the practices of our hospitals are, and the sufferings *patients are in the habit of enduring at their hands*, to feel for this man, or for any other man placed in a similar situation!" Canting hypocrite! this from the man who in the very case before us published his account of the patient's sufferings in the form of a drama, mingled with low and scurrilous gibes. Mark also the following—"Give such a verdict as shall satisfy the *poor*—as will shew that men are not to go into offices of this sort where the *poor are to be killed*, and that they shall not be at liberty to wade and ride through blood up to their necks to eminence in their profession. Ah! gentlemen, give such a verdict as shall satisfy the *poor*!"

There is in the various paragraphs we have quoted, more deep-rooted malice—more abandoned violation of the truth—more hypocrisy—and more open outrage of common sense, decency, and feeling, than we ever remember to have met with, even in the pages of the Lancet. It is now quite obvious that Wakley, alarmed at the change which has taken place in the estimation of his work within the last few months, finds it necessary to adopt a new system; he therefore throws off the mask, and openly declares himself the champion of the ignorant and illiterate^[22]—conjuring up to their imaginations evils which do not exist—denouncing the medical profession as one of blood and murder—medical men as "thriving and fattening on their fellow-creatures"—and calling upon the "humane contributors to our hospitals and infirmaries" to support him.

Why really now, if the times of political turbulence were to return, and radical reform again to come into vogue, Cobbett and Hunt would have a most valuable coadjutor; but as it is, we almost think the sphere he has chosen is too limited for such a genius. Humbug, to be successful, ought not to be very open to detection; alleged abuses ought not to be of a kind which

every man can satisfy himself are entirely without foundation; and he who professes disinterested motives ought not to begin by raising a subscription for himself. We offer these few hints to Wakley, in hopes they may be of use to him in his new career; above all, we advise him to be sparing in protestations: there are some already who look upon his sincerity with a suspicious eye; for although it is no doubt true, that he would "rather die in a dungeon, and expire in a ditch," than injure any man, yet somehow an idea has got abroad, that his constant habits of lying and slandering are not quite in keeping with these assertions. Nay, we have heard of some even of the "humane contributors" to our charitable institutions, who have been so unreasonable as to disbelieve all Wakley's stories about the *cannibalism* of hospital surgeons, and to look upon the whole system of the Lancet as one of knavery and imposition.

WAKLEY'S SUBSCRIPTION MEETING.

A most amusing scene took place at the Freemasons' Tavern on Tuesday evening, of which we subjoin an account. The object of the meeting was, "to present Mr. Wakley with some decided mark of their approval of his spirited conduct in his late trial;" or, in other words, if possible, to raise a little money, and bolster up the Lancet a little longer. The persons present were small in number, but great in zeal; and as much noise, hissing, laughter, and shouting, took place as would have done credit to a much larger assembly. As this is a merry season, and it may amuse our readers, we subjoin the speeches of the principal orators on either side: it will be perceived that both the Gazette and Lancet had some handsome compliments paid them; and that Wakley got thoroughly abused even by some of those who went to the meeting as the champions of reform. There was not a single individual present of any weight in the profession, and the chair was occupied by a young man wholly unknown, except as secretary to the meetings held two years ago on the subject of surgical reform. Could no one whose name might sound familiar in the public ear be prevailed upon to go?—could none be found but Pateys, Wallers, Mills, Elliots, Childs, and the Lord knows who? Where was he, the idol of the Lancet, who on other occasions graced the chair—where he who first ycleped that Journal the "invaluable"? Alas! poor Wakley, what an exposure of a sinking cause! even thy former friends tremble for their reputations, and desert thee.

SURGICAL REFORM.

A meeting was held yesterday evening at the Freemasons' Tavern, in pursuance of an advertisement calling on the "Friends of Surgical Reform, the Supporters of a free Medical Press, and the humane Contributors to our Hospitals and Infirmaryes," to meet there, in order to adopt measures for presenting Mr. Wakley, the Editor of the *Lancet*, with some decided mark of their approval of his spirited conduct, in his late trial in the Court of King's Bench, at the suit of Mr. Bransby Cooper, and of the principles which he so powerfully advocated on that occasion.

[Mr. Patey, who was called to the chair, stated the object of the meeting; but as his speech was rather long, and contained nothing of the least interest, we omit it.]

Mr. Waller, in proposing the first resolution, adverted to the terms of the advertisement, and remarked that the question for them was not whether they would support Cooper against Wakley, or Wakley against Cooper; but whether they would have a free medical press or not—(cheers). Every body must admit that there was only one medical publication that was entitled to be considered free—only one that dared to tell the truth. Another had been attempted to be set up against it, but this wretched rival was gagged and manacled, and its reports were only a medium which distorted truth, and were almost unintelligible. It was not, however, to support any particular man, or publication, that he came to that meeting, but to support the general cause in which they were all interested. He concluded by moving, as the first resolution, that "the best interests of the medical profession and of the public are identified with the cause of medical and surgical reform; and that Mr. Wakley, as Editor of the *Lancet*, having given the first impulse to that cause, and having subsequently advocated it with undeviating firmness and fidelity, is entitled to the cordial thanks and support of this meeting."

Mr. Mills seconded the resolution.

Dr. Shiel said this was an important meeting, in his opinion; for, if this resolution were passed, it would identify the medical profession with the Editor of the *Lancet*. If the principle embodied in that resolution went forth to the world, it would be attended with consequences deeply injurious to the profession. He knew neither Mr. Bransby Cooper nor Sir Astley, and therefore he was perfectly impartial. The first question was, how far the medical press was conducted with advantage to medical science and the members of the profession. Now he contended, but with great respect, and disclaiming all personal hostility, that the *Lancet* had nowise contributed to the progress of medical science or the improvement of the profession—(loud hissing, and some cheers). He trusted that the meeting had not been got up by a few of the friends of Mr. Wakley, and he was sure that that gentleman was too respectable and too dignified in his proper person to need any such attempt—(applause). He trusted that the meeting was before the British public to ascertain how far the *Lancet* was useful or advantageous to medical science; and he hoped that, however partial the gentlemen present might be to Mr. Wakley, they would be governed in what they should do by the real interests of science—(hear, hear). The question was one of great interest; for it was one that embraced not only the freedom of the press but the advancement of medical science. With respect to the freedom of the press, they lived in a country where the law always maintained that principle. When Mr. Abernethy had applied for an injunction against the *Lancet*, Lord Eldon said that there could be no doubt that the *Lancet* was a most useful publication—(applause). He did not know whether that observation had been brought forward at the late trial; but taking the case—what was it? It appeared that the operation was one of peculiar difficulty, as stated upon oath by some of the most experienced surgeons of the day. This operation was laid hold of by the *Lancet*, and published not as a medical but as a tragical performance, in which all persons without science or knowledge were appealed to, and no opportunity afforded to Mr. Cooper to vindicate himself. Here, then, was a low and pitiful attempt made through the medium of malice and calumny (loud hisses and uproar). Here was a rude and

pitiful attempt by malice and calumny (renewed signs of disapprobation). And yet it was contended that this had advanced the medical profession. The evidence on the trial went to shew that the dangers by which the operator had been surrounded were not small, and that there was no just reason to accuse him of unskilfulness in what he did; and was it then to be said, in the face of the British public, that a Meeting had been held at the Freemasons' Tavern to offer praise and reward to the individual whose conduct had, by the verdict of the Jury, been stamped with the accusation of untruth (hear, hear! and hisses)? He would not deny that there was a degree of talent displayed in the *Lancet*; but neither could it be denied that it was a public depredator by means of reports surreptitiously obtained; and the title of "literary raven," which had been applied by Sir James Scarlett, appeared to him to truly depict its character (great hisses and uproar). Was it to be contended that any one had a right to come into a private lecture-room, and catch up all he could hear, and then to publish what he had thus surreptitiously and fraudulently obtained (immense uproar and hisses)? As far as the medical interest was concerned, he looked upon the *Lancet* as one of the most injurious publications that had ever appeared (hisses). All the best writers on the subject had dwelt with peculiar emphasis on the necessity of secrecy among the members of the profession, but the *Lancet* broke through this good rule, and made public whatever came within its notice; and he had no doubt that if it could get at the cases that occurred in private practice, it would give them without any scruple (uproar).

The *Chairman* begged to remind the speaker that at all events Mr. Wakley had not as yet done so.

Dr. Shiel.—I am arguing on the principle which appears to actuate Mr. Wakley.

Mr. John Elliot rose to order. He could not allow Dr. Shiel to be going upon suppositions; the thing that he supposed had never been done.

Dr. Shiel was sure that whatever turn might be given to the subject in England, that at all events the conclusions that the Meeting appeared to be coming to would be resisted abroad. The weight of Sir Astley Cooper's name abroad would overpower any such attempt, and, in fact, would render it contemptible; so that any attempt to prop up so low, pitiful, and mean an attack, must inevitably be attended with disgraceful failure (hisses).

A gentleman observed, that he was much surprised at what had fallen from Dr. Shiel. He had been induced to suppose that the *Lancet* possessed talent, as he had heard it praised in so many directions, and he certainly had never heard till that evening that it was only to be found in the hands of the illiterate (hear, hear! and a laugh). He had always understood that it was only destined for the use of students and other members of the profession, and that it was likely to be of great use in reforming the abuses of the medical practice (hear, hear! and cries of "So it will!"). For his own part, he was glad that medical men were occasionally cut up, because when they did what was wrong it was proper that they should be told of it; and when they did what was right, it added a fresh stimulus to their exertions (applause).

Mr. Thomas observed, that he had not intended to say any thing at the present meeting; but as Dr. Shiel, in the course of his speech, had thought proper to make some remarks tending to calumniate Mr. Wakley's witnesses on the trial, of which he was one, he felt called upon to say a few words (hear, hear!). Dr. Shiel seemed to consider them all as the mere scum of the earth—as a miserable band, collected together to support Mr. Wakley, whatever might be the consequence; as though he was desirous of going the whole length of the *Medical Gazette*, which stigmatised them for what it called "their deep-laid contrivances—their rankling enmities, and their bitter revenge." For himself he could say, that it was not till the evening previous to the trial that he had been supœnaed, and that he had previous to that time never seen Mr. Wakley.

Dr. Shiel said that he could not have alluded to the gentleman who was speaking, as he did not even know his name.

Mr. Thomas then went on to observe, that as Dr. Shiel appeared to be the advocate of the opposite party, he also supposed that he was a contributor to the aristocratical (*Medical*) *Gazette* ("No, no!"

from Dr. Shiel). If he were not, at all events he had put forward ideas that were quite consonant to the sentiments expressed in that Journal. In giving his testimony, he had not spoken of the instruments employed in the operation, but of his own impression on the subject, having been present thirty-five minutes; and that impression certainly was, that the operation had been performed in a bungling and unscientific manner (loud applause, mingled with hisses). Was he then, because he happened to be a witness on the occasion, to be put down, pell-mell, by the unfounded imputations of any one? The highly-principled, honourable-minded Medical Gazette, that had determined never to admit any personalities, had loaded Mr. Wakley and Mr. Lambert with all sorts of abuse. Was this what they intended to call acting on principle (applause, and cries of "No, no!")? He certainly had heard that Mr. B. Cooper was an amiable man in private life; but what had that to do with the question at issue? What had they to do with the private character of a man in a public office (cheers)? Surely the witnesses of Mr. Wakley were as competent to speak of the manner in which the operation was performed as those who had not been present (applause); and as he had seen many operations performed he conceived that he was a competent judge of the skill of the operator (applause).

A gentleman, who appeared to be a student, thought that the introduction of any attack upon the witnesses, or indeed of anything that occurred at the trial, was irrelevant (hear, hear!). The way to answer a speaker was not by hisses, but by disproving his arguments and rebutting his facts (applause). If questions were to be settled merely by strength of lungs, he was afraid that instead of the right side prevailing, success would uniformly attend those whose physical powers probably much exceeded their mental (laughter and applause). He could not agree with the gentleman who had stated that the law of England was the protector of the freedom of the Press; for it was well known that the Judges had decided, over and over again, that truth was a libel (applause). It was only the vehicle that conveyed public sentiment, and not the force that impelled it forward. He was convinced that any attempt to connect medical reform with Mr. Wakley should be studiously avoided and disclaimed. As a friend of free discussion in every case, and anxious to uphold the principle, whatever he might think of the instrument, he intended to propose, as an Amendment, "That the latter part of the Resolution, relating to Mr. Wakley, should be omitted." This would save the cause of medical reform from being identified with Mr. Wakley, of whose impartiality he would give a specimen. In the last number of the *Lancet*, the report of the late trial was given from the *Times*, but with a remarkable omission. The Lord Chief Justice had rebuked one of the defendant's witnesses for not answering the questions in a straightforward manner, and this passage was omitted. If Mr. Wakley would do this on such an occasion, would he be more candid when the public eye was less upon him? He, therefore, wished to have the question of medical reform kept separate from Mr. Wakley individually. The first part of the Resolution would be carried without a dissenting voice, as on that principle they were all agreed, whatever they might think of the conduct of individuals.

[One or two persons then made speeches for or against the amendment; which was negatived, and the original motion carried.]

Mr. Hensley then, after observing that Mr. Wakley's conduct was highly praiseworthy, because it tended in every way to the benefit of humanity, moved the second Resolution, which was to the following effect:—

"That the purposes for which the Hospitals and Infirmaries of the Metropolis were founded, and that the views of the humane contributors to their funds, are materially promoted by the weekly publication of reports detailing the medical and surgical treatment of the unfortunate patients; and that Mr. Wakley having originated the practice of publishing Hospital Reports, has conferred important benefits on Medical Science, and on the cause of humanity."

The resolution was then carried unanimously.

Mr. John Elliott, on moving the third resolution, said that he did not come there to interfere in the quarrels of Wakley and Cooper. They had acknowledged that Mr. Wakley's exertions had been very

conducive to medical reform, by the last resolution they had passed. Indeed, there could be no doubt that he had greatly served the cause of humanity, by preventing idleness on the part of medical persons, and compelling attention to the poor placed under their care. It was his opinion that the editor of the *Lancet* ought to be indemnified for the whole expense entailed on him by the late trial, including the damages awarded against him. This would not be a private subscription, like Brodie's, but one open to the world, and not to be questioned in a court of justice. He would not support Mr. Wakley if he had attacked private character; but he would, as the editor of the *Lancet* and the representative of the medical free press. He concluded by moving—

"That the independent and impartial principles on which the *Lancet* was first established, have been preserved by Mr. Wakley at all risks; and as it was acknowledged at the late trial, that the legal expenses of his opponents on another occasion have been defrayed by certain hospital physicians and surgeons^[23], it is farther resolved, that a subscription be opened for the purpose of defraying the expenses of the late action."

[Dr. Shiel here spoke very strongly against entering into a subscription—while some spoke in its favour.]

A discussion then arose about the question of whether an account of a second operation for lithotomy by Mr. Cooper (in which he had been successful) had been sent to Mr. Wakley? It was stated that such was the case, but as the action for the present libel was then commenced, *Mr. Wakley thought that he perceived something in that second report that might tend to aggravate the damages, and which, at the same time, would be farther injurious to Mr. B. Cooper.*

The third resolution was then carried by an immense majority, only five hands being held up against it.

Mr. Evans moved, "That the statement of professional facts in an unprofessional form, such as the dramatic instead of the narrative, is highly reprehensible in the individual, and detrimental to the best interests of science."

This resolution, after some turbulent discussion, was withdrawn.

Mr. J. L. Stevens then moved, as the fourth resolution, that "In accordance with the feelings this meeting has expressed, Mr. Wakley be invited to attend a Public Dinner, and that a Committee be forthwith nominated to arrange the same."

Mr. Rogers seconded the resolution, and it was carried unanimously.

The members of the committee were then named, and power given to them to add to their number.—The chairman then vacated his office, and the meeting separated.—*Morning Chronicle, Wednesday.*

In the course of the evening, the conduct of one of the speakers gave for a short time an amusing turn to the discussion, which for a great part had appeared to many extremely dry and tedious. The gentleman in question, (not a medical man), in order to show the opinion he had entertained of the object of the meeting, proposed to read for the audience a letter which he himself had written to the chairman of the committee, asking permission to attend. He began, but he had scarcely got through a few words when he declared he was not able to make out his own writing, and requested the chairman to assist him. The chairman made the attempt, but was not more successful. Another gentleman then undertook the task, but with no better effect. The audience received each unsuccessful effort with loud laughter, which so much annoyed the writer that he took back the letter, and again tried to go through with it, but not being able to make it out, he proposed to read for the meeting two letters which he wrote on the same subject to the editor of a Sunday paper. (Loud laughter followed this proposition, which was increased, when, on an attempt to read one of them, he

had no more success than before.) The gentleman, after complaining of the want of courtesy in the meeting, resumed his seat, declaring that he would give the letters to the reporters.—*Times*, *Wednesday*.

HOSPITAL REPORTS.

GLASGOW ROYAL INFIRMARY.

Injuries of the Head.

D. M'Millan, æt. 40, seaman;—while he was employed in the hold of a vessel, a heavy iron block, of a round form, fell from the rigging, a height of 20 feet, and struck him on the head. He was knocked down and stunned, but was able to rise and to walk with a little support. He was brought about a mile and a half in a hackney-coach to the hospital, and admitted by Mr. Cowan at one o'clock P.M., half an hour after the accident. At this time he had a stupified look, but was quite collected, and was able to walk across the ward; breathing and pupils both natural; pulse 75, of moderate strength. There was a wound in the scalp, commencing nearly over the centre of the sagittal suture, and running about three inches backwards and towards the left; along this a portion of skull, corresponding to the wound in size, was felt fractured, detached, and irregularly depressed. His right arm was benumbed and nearly powerless, but no mark of injury was observed on it.

It was determined to remove the detached piece of bone, and the original wound being extended lengthwise in both directions, an incision was made through the scalp at right angles to it over the left parietal bone, thus bringing the fractured portion into view. This was found to be broken down into several fragments, some of which were firmly impacted under the surrounding cranium, and one spiculum, driven through the dura mater, had penetrated the brain. The trephine was applied a little to the left of the fracture, and the intermediate part removed with Hey's saw; the detached pieces of bone, ten in number, of various sizes and irregular shapes, were removed with the forceps, the scalp replaced and retained in position by a stripe of adhesive plaister and very light dressings. During the operation one artery required ligature, and about $\frac{3}{4}$ v. of blood were lost: he had lost perhaps as much before. He remained quite sensible, bore it well, and shortly afterwards had half a glass of port wine in warm water.

Ten P.M.—Has been tolerably easy; complains of pain of fore-part of head; pulse 82, firm.

Fiat V. S. ad 3xviii.

Sumat statim Submur. Hydr. gr. viii. Pulv. Antimon. gr. vi.

Applic. Capiti Lotio ex Alcohol. et aq. calcis.

Second day.—After bleeding, thought feeling of numbness of right arm somewhat lessened, and to-day has rather more power of it. Passed a tolerable night; a little head-ache; countenance pretty natural; tongue slightly loaded; thirst; no stool.

Sum. stat. Sulph. Magnesiae 3j. et rep. vesp. si opus.

Third day.—From an aversion to use the bed pan, he rose to stool last night, and fell forwards on the floor, seemingly from having entangled himself in the bed clothes; he got into bed without assistance; had a shivering fit shortly after, but has had a pretty good night; bowels freely opened; head-ache slight; pulse 84, soft; tongue dry in middle. Towards the evening of this day he appeared drowsy; and on the fourth day he is reported as seeming confused and uneasy; countenance anxious; articulation difficult; the right arm was more paralysed, and indeed the whole right side seemed now to partake in the paralysis; a small part of the wound had adhered, the remaining part was flabby, discharging thin serum. In the evening the difficulty of articulation and drowsiness had increased. Twelve leeches were applied to each temple, and in a short time he seemed a little livelier.

Fifth day.—Much in the same state; took a little beef tea.

Sixth day.—Paralytic symptoms increased; breathing laborious. Blister to the neck. It may be worthy of remark, that in the few words he now spoke he never made use of the English language, but of the Gaelic, which was his native tongue; and even when told that he was not understood, he persisted in repeating the Gaelic phrase.

Seventh day.—A small fungus was observed in upper part of wound, to which lint dipped in lime water was applied, and moderate pressure. The fungus continued to increase.

On the eleventh and twelfth days he had slight bleedings from the scalp, which stopped spontaneously. Stupor, &c. greatly increased, but he still recognised his friends; took a very little chicken soup or beef tea daily, with lemonade, &c. for drink.

On the evening of the thirteenth day bleeding to the extent of $\frac{3}{4}$ iv. or v. took place from fungus: it was checked by application of saturated solution of alum, but he sunk rapidly, and died that night.

Inspection 36 hours after death.—The fungus protruded considerably beyond the scalp, and was rather larger than a hen's egg, of a dirty brown colour, and a soft spongy consistence; it completely filled up the opening made in the skull by the trephine and removal of detached bone. The pericranium to the inferior side of this opening was found detached from the bone, to the extent of a crown piece, and beneath it the bone was rough, and covered with thin purulent matter. The dura mater, for two inches around the fungus, was covered with pus; this membrane, by sloughing, had allowed the fungus to protrude, and its edges adjacent to the aperture were thickened. On removing the dura mater, the fungus was observed to arise, partly from the middle, but chiefly from the posterior lobe of the left cerebral hemisphere: it occupied a space about three inches in length, and an inch and a half in breadth, extending to within a line or two of the roof of the ventricle; at its anterior part was an abscess, containing $\frac{3}{4}$ ss. of pus. The fungus seemed to be a degeneration of the cerebral substance; sections of it showed the cerebral matter first dotted with an unusual number of bloody points, then assuming a greyish colour, which gradually passed into a dirty brown. With the exception of these bloody points, the brain presented no unusual vascularity: it was quite firm, except in the immediate neighbourhood of the fungus, where it gradually became softer as the colour of the cerebral substance became deeper, till in the centre of the fungus it was nearly of a broken down consistence. A quantity of serum was found in each ventricle.

Another man who had been struck by the piece of iron which fractured M'Millan's skull was brought up to the hospital at the same time. He had received the blow on the vertex of his head. On admission he was in a state of fury, requiring the efforts of several men to hold him. His head was bruised out of all shape, quite depressed behind, and a fracture also of the frontal bone; there were evident symptoms too of fracture of the base of the skull. The case was hopeless. Several large depressed and detached pieces of bone were removed, but he died a few minutes after the operation.

During the same week an old woman was brought up to the infirmary, who, the day previously, had fallen headlong down a stone staircase. She laboured under the severest symptoms of concussion, and besides had a comminuted fracture of the humerus into the elbow joint, and of the radius into the wrist joint of left arm. She was past all treatment, and died on the second day. The case is mentioned chiefly to remark, that although no external injury of the head could be observed, on inspection a fracture was found extending completely across the anterior part of base of cranium.

There are no dispensaries in Glasgow, but such of the poor as are unable to obtain admission into the Royal Infirmary, and those who are not so ill as to submit to the confinement of an hospital, or for other reasons prefer remaining in their own houses, are prescribed for, and, if necessary, visited at the public expense. For this purpose the city is divided into districts, and a surgeon appointed to each. The "district shop" thus resembles a dispensary, where the surgeon prescribes in the presence of his pupils, who indeed, under his superintendence, have the management of many of the cases, both among the patients who receive advice at the "shop," and those who are visited at home. Although under the care of a surgeon, the diseases treated are both medical and surgical, for there is little distinction made between the two branches of the profession here.

The following case occurred under the care of Mr. Stirling:—

W. M. æt. 28, while returning home during the night in a state of intoxication, fell from a height of ten feet, into a court paved with stone. He was rendered insensible for some time, but on recovering was able to crawl to the nearest door, and was conveyed home. Medical assistance was immediately procured. On the forehead, about an inch above the nasal process of the frontal bone, was a cut an inch and a half in length, extending obliquely downwards from the right side; through this a triangular fracture was felt, the base of which was in the direction of the external wound, and the apex pointing downwards; the fractured portion of the bone was slightly depressed; the bones of the nose were also fractured, and there was great discharge of blood from the nostrils. One third, or nearly, of the upper lip was cut off, and a good deal of blood had been lost from the labial artery; the lower jaw at the symphysis was fractured and comminuted, and several pieces were perceived to be loose. Was quite sensible, remembering every thing up to the period of the accident. Pulse 80; has vomited frequently.

Wound of forehead was dressed with adhesive plaister; the loose pieces of bone were removed from the lower jaw, and it was afterwards retained in juxta-position by a chin bandage.

Early next morning he was bled to 3xii. and had a purge of jalap and calomel, by which his bowels were freely opened. That night he felt considerable pain of head; pulse 90, full, and strong. He was bled again to 3xxv. Blood first drawn had the buffy coat. After the bleeding the pain of head diminished, and he continued to do well. When the swelling and tenderness of face had subsided, it was observed that the fracture communicated with the frontal sinus. On holding his nose and attempting to blow it, matter and air escaped by the fracture. The man was anxious for a speedy cure, and for this purpose an operation to remove the depressed piece of bone was at one time thought of; there was no constitutional disturbance, however, and the case was left to nature. The discharge gradually diminished, and the wound healed up in about a month. The bones of the nose retained their position, and the broken ends of the jaw, after the swelling had subsided, were kept in their proper situation by a pasteboard splint till re-union took place. The lip also healed easily, and no deformity is now observable.

GUY'S HOSPITAL.

Inflammation of Hernial Sac.

The previous report of this case (Gazette, No. 51) conducted it to Nov. 18th, the tenth day from the operation, and the third on which fæces had been discharged from the wound; that discharge continued on the 19th, but on the 20th pus only was seen.

Nov. 22.—Yesterday the dresser, Mr. King, observed a portion of *solid* fæces make its escape, and to-day a similar discharge, in the fluid form, is manifest.

Nov. 26.—Since the 22d, nothing but healthy pus has been seen; the wound is rapidly healing, and the patient's general condition as good as can be desired.

Dec. 4.—He has begun to wear a truss, and is to-day allowed to leave his bed.

Dec. 8.—As regards his original complaint, the man is perfectly convalescent, but he now discloses a small abscess in the scrotum, which has been gradually forming, and will detain him in the house some time longer.

If it be a sound surgical maxim that, in strangulated hernia, the patient's safety depends upon an early operation, it is also a maxim no less sound, that where the evidence and symptoms of strangulation are imperfect, or dubious, the benefit of the doubt should be given to the operation, and it should be performed: for it is better that buboes should be laid open, or inflamed sacs cut into, twenty, aye, fifty times in succession, than that in one case a man should be suffered to die with strangulation of the intestine unrelieved.

These remarks apply strikingly to this case of Mr. Key's, in which, although no hernia was found, they perfectly justify the operation. No case of strangulation could be more distinctly marked, or more clearly call for the knife, than did this; and had Mr. Key refused to use it, he would have deserved the scoff which, by a contemporary, has so preposterously been thrown on the "no hernia case." But, more than this, the operation was not only justifiable, but beyond a doubt *beneficial*—beneficial inasmuch as it at once relieved the distress of the patient, secured him from more extensive suppuration, from farther sloughing of intestine, and probably from extravasation of fæces into the abdomen; rendering also the process of reparation more easy and rapid.

Hereditary Hare-Lip.

Dec. 4th, Mr. Key performed the operation for hare-lip on a child a few months old, whose case was remarkable only as it afforded an instance of *hereditary* malformation. The father of the child had congenital hare-lip, for which no operation had been performed, and of his six children *two* were born with the same malformation.

Queries.—1. If the father's lip had, in his infancy, been restored to the natural form, would the malformation in that case have descended to his children?

2. Is the casualty in these cases to be traced *directly* from the father to the incipient embryo, or *indirectly*, through the medium of the mother's

imagination—It has been said that paternal resemblances are produced in the latter mode. G.

PARIS HOSPITALS.

Hôtel Dieu.

Cases of Gangrene of the Lungs.

Some interesting cases of gangrenous suppuration of the lungs were published by Dr. Chambers, about eighteen months ago, in which he particularly pointed out the horrible fœtor of the breath as a diagnostic mark of the disease. The following cases which recently occurred at the Hôtel Dieu are good illustrations of more extensive destruction of the lung by a similar disease.

CASE I.—A man, aged 32, of good constitution, laboured under bronchitis for some weeks, to which he paid little attention, when he was suddenly seized with pain in left side, dyspnœa, considerable cough, and a rigor, followed by fever. He was admitted into the Hôtel Dieu next day. Crepitation was perceptible over the whole extent of the left lung, posteriorly, and reaching to the site of the pain in the side, which was increased by percussion; great oppression, with frequent cough, and expectoration of violet-red colour, mixed with mucus; pulse frequent and small; constant chilliness; cheeks flushed.

Next day the symptoms remained unabated, and *fœtor of the breath* was observed. Two days after it is stated that the expectoration maintained the same appearance, but *was now fœtid*; the breath was, however, still more offensive, and during the fits of coughing particularly so. In the evening the breath is said to have become *gangrenous*.

21.—The cough was small, difficult, and painful, producing an expectoration of violet-coloured matter, approaching to chocolate, *with a characteristic odour*.

22 to 25.—Expectoration and breath emit an *intolerable stench*. On the latter day the patient died.

Examination.—The body not emaciated. Strong adhesions of the left lung to pleura costalis. At its posterior part a large gangrenous layer, occupying

the two superior thirds of the lung, covered at some points by a false membrane. The lower third, and the parts round the gangrenous portion, in different degrees of inflammation, and hepatised; the putrid mass was in great part black or violet-coloured, containing fragments of the pulmonary texture; the smell gangrenous, but less so than that of the breath during life; pus might be squeezed from the adjacent part of the lung.

CASE II.—A man, aged 55, enjoyed good health till the beginning of May, at which time he experienced pain in the left side of the thorax after exposure to cold. During the next few days the cough was frequent, and the expectoration tinged with blood. A rigor now came on, followed by a distinct exacerbation of fever, and he came to the Hôtel Dieu. At this time he presented symptoms of inflammation of the lungs, and was bled with relief; he left the hospital in twelve days, being sufficiently well to resume his occupation. Scarcely had he done this, however, when he had a relapse, accompanied by great lassitude, oppression of breathing, and frequent cough. At the end of a week he returned. He was now at the twenty-first day of the attack. His skin was yellowish; face pale or of leaden hue, and greatly altered; his cough frequent, with abundant expectoration of matter like chocolate, with small specks resembling pus, and little bodies about the size of peas, which appeared to be portions of the lung. This putrid mass exhaled a *gangrenous odour*, and the expired air was impregnated in a high degree with an equally disgusting smell, which surrounded the patient's bed with a contaminated atmosphere. Percussion gave a clear sound over the whole extent of the chest; auscultation did not discover the pulmonary expansion on the right side. The pulse was weak, but not frequent; the patient was in a state of extreme prostration. Next day the odour of the breath and expectoration was, if possible, more dreadful. He lingered two days longer, during which time the fœtid smell somewhat diminished, and the proportion of pus in the expectoration increased.

Examination.—The left lung had contracted firm adhesions, particularly above and behind. It was torn in attempting to remove it, and there issued from the middle and posterior part a large quantity of dark-coloured matter, similar to what had been expectorated. An incision was made along the back part of the lung, and laid open a large cavity, occupying all the extent of the pulmonary organ, and still in part filled with the same kind of dark putrid matter. On washing out the cavity it was perceived that it was lined

with a smooth and white membrane, to which were still adherent, at some points, dark filamentous shreds; these were removed by the least touch, and left the surface smooth beneath. The boundary was formed by a false membrane, about a line in thickness, which separated the mortified from the sound parts. The lung was reduced to a kind of bag, the parietes of which were on an average about an inch thick. In the upper part of the lung, which was hepatized, an incision discovered a number of smaller tubercles, which in the centre were in a crude state; and at one point several had suppurated, forming a cavity large enough to contain a nut.

[The Editors of the *Journal Hebdomadaire*, from which these cases are abridged, add, in a note, that although the fœtor of the expectoration, such as above described, is usually dependent on gangrene of the lungs, yet that this is not invariably the case. In confirmation of this assertion they refer to three instances of fœtid expectoration—in the first, the patient is still alive; in the second, there was chronic bronchitis, with considerable dilatation of numerous bronchi; and in the third, bronchitis with some appearances of chronic pneumonia. Laënnec gives a case of dilatation of the bronchi, with remarkably fœtid expectoration. The argument derived from the first patient having survived we consider to be altogether erroneous, as patients more frequently recover from this than other forms of purulent expectoration.—ED.]

PROCEEDINGS OF SOCIETIES.

WESTMINSTER MEDICAL SOCIETY.

December 6th, 1828.

JAMES ARNOTT, ESQ. IN THE CHAIR.

Pursuant to announcement, Mr. MacKelcan introduced the subject of concussion of the brain. After remarking on the several symptoms, and mentioning the other affections of the head with which concussion is apt to be confounded, Mr. M. directed the attention of the society more particularly to the treatment in the stage of collapse, adopted by Professor Andrini of Florence.

Dr. Duffin stated, that having been in Florence, he had witnessed the employment of three, four, or six ounce bleedings employed by the Professor in the stage above-mentioned. The principle on which they were used was that of their acting as a stimulus to the brain; and really the plan was not apparently attended with any bad effects. When re-action has ensued, the practice of M. Andrini is the same as that resorted to in this country.

The discussion then wavered between apoplexy and concussion, and ended by some good-humoured sparring between Dr. Addison and Mr. Gilbert Burnett, on the capability of the brain containing more blood at one time than another.

The President announced from the chair that a special meeting would take place on Friday evening, December 12, at 7 P.M.

December 20th, 1828.

CÆSAR HAWKINS, ESQ. IN THE CHAIR.

After some business had been dispatched, to which we shall allude in another place, Mr. Jewel related the particulars of a case of "supposed extra-uterine fœtation." The case, which is one of much interest, is as follows:—

Mrs. F. a robust, healthy woman, ætatis 27, had been married one year, and aborted once; when, in the month of September 1827, she quitted her

husband in London, and accepted a situation in the country. After the expiration of a few weeks she received permission to come to London for a day; and, naturally enough, passed that night, (November 8th), with her husband, returning again to the country next morning. Soon afterwards, the morning-sickness, heart-burn, and other sympathetic affections of pregnancy, appeared; but the catamenia still returned at the regular periods, though scanty and paler in colour than natural. She now quitted her situation, returned to her husband, and engaged a midwife to attend her in her expected confinement. The circumstance of quickening occurred in the latter part of February, and was accompanied by a severe paroxysm of fainting, whilst the abdomen gradually enlarged, and the motions of the child (as was imagined) could not only be easily detected with the hand, but were visible to the eye. On the 9th of August, making a period of 274 days, she was seized with the usual premonitory symptoms of labour; the midwife arrived; found her walking her chamber, and concluded that the process had considerably advanced. Three distinct uterine contractions took place subsequent to the midwife's arrival, the last of which was so severe as to compel the patient to grasp firmly the back of a chair, and induce, after its subsidence, excessive faintness. From this time no further parturient effort was made; no examination per vaginam was instituted; neither was there noticed any sanguineous vaginal discharge. When Mr. Jewel saw her, examination detected nothing unusual in the os uteri, and the symptoms she presents are these. The abdomen is about the size of a woman's in the sixth or seventh month of pregnancy, the enlargement being general. Her general health is much disturbed: she complains of pain in the left hypogastric region, particularly when the abdomen is compressed by the hand or stays—and she also complains of weight in the abdomen, especially on leaning forwards. The woman herself has a strong prepossession that "something is to come away," or that she might be relieved by an operation. The woman believes that she still feels the motions of the child, though feebly.

Having read this case, Mr. Jewel concluded by offering some observations on the subject of extra-uterine fœtation. He considered this as probably an instance of "graviditus abdominalis," and remarked that the uterus not having expelled its *dezidua*, was no conclusive proof to the contrary, Dr. Blundell and Mr. Langstaff having examined cases where this was wanting.

Dr. Locock never saw a case where laceration occurred in the early months of pregnancy. Dr. L. remembered the particulars of a case where the uterus was lacerated in the fourth month, and the ovum escaped into the cavity of the abdomen, and the symptoms of pregnancy were exceedingly strong. Dr. Locock almost doubted whether Mr. Jewel's was an instance of pregnancy at all, for frequently women enjoying sexual intercourse have enlarged abdomens, and many of the other symptoms of pregnancy, which a few doses of purgative medicine will remove.

Two remarkable instances of extra-uterine foetation were related by Mr. Burnett and Mr. Cæsar Hawkins; and towards the conclusion of the evening the discussion turned on the powers of the ergot of rye. Dr. Locock had frequently tried it in amenorrhœa, and, on the whole, without any very good effects. In one remarkable case, where the lady particularly wished to have children, Dr. Locock having boiled an ounce of the ergot in a pint of water down to half a pint, gave an ounce and a half of this decoction three times a day. After the young lady had taken about half a pint, the most violent convulsions succeeded, which placed her life in imminent hazard, and from which it was long before she completely recovered. She menstruated twice after that, but then the discharge again was arrested, and has never since returned, nor has she become pregnant.

The practice of "bougieing the uterus" in cases of dysmenorrhœa was brought upon the tapis, and excited a good deal of laughter, the members appearing to consider the proposal ridiculous in the extreme.

MR. LAMBERT.

At the last meeting of this Society it was proposed, on the recommendation of the Committee, that Mr. James Lambert should be expelled; and after some discussion it was agreed that the sense of the members should be taken by ballot at the next meeting, Saturday, January 3, 1829. It was thought that proceeding at once to his expulsion might give the appearance of the measure originating with a party, and that the delay would enable the deliberate judgment of the whole Society to be passed upon his conduct.

HUNTERIAN SOCIETY.

Dec. 10, 1828.

DR. BILLING, PRESIDENT, IN THE CHAIR.

Mr. Leese, Jun. exhibited to the meeting a specimen of medullary sarcoma, taken from the forehead of a man after death, with a portion of the os frontis. The man was fifty-eight years of age, long asthmatic, and appeared to die of hydrothorax. He never suffered any pain in the tumor, but expressed a sense of distention. It was immovable at the base, and there were some fissures in the os frontis, from caries. The dura mater at the corresponding part adhered firmly to cranium, and on its being removed some spiculæ from the carius inner table remained adhered to it. There had never been any symptoms of cerebral affection. A tumor of the same nature had formed on the scapular extremity of the clavicle, and had occasioned anasarca of the limb.

Mr. Key supposed that the tumor had originated from the diplœe, and believed that the only chance of relief in such a case would be trephining; but he acknowledged that there was little encouragement to operate in these cases, owing to the tendency of the disease to return in some other part. He related an instance of the true medullary fungus taking its origin from the cancellous structure of the os femoris. The limb was removed, but the patient died. A circle of fungoid glands was observed at the base of the heart, and some of the bronchial glands were in the same condition.

Dr. Conquest expressed his belief that carcinoma was always a constitutional affection, and stated that at least in nine cases of cancer uteri out of ten, there was carcinomatous affection of the pylorus, mesenteric glands, or liver.

Mr. Key and Dr. F. Ramsbotham had not observed, in their examinations, the coincidence noticed by Dr. Conquest.

The remainder of the evening was occupied by a discussion on the different kinds of ulceration that occur about the os and cervix uteri, and the circumstances under which the removal of the uterus, or the amputation of the cervix, may be warrantable.

Dr. Babington adverted to the occasional existence of a family disposition to carcinoma. He had known three sisters successively die of cancer in the breast.

MEDICAL SOCIETY OF LONDON.

Dec. 13th and 20th, 1828.

DR. HASLAM IN THE CHAIR.

The subject of discussion, at the first of these meetings, was stricture of the rectum, when the diagnosis and treatment of carcinoma of that part came under consideration. There was a remarkable discrepancy in the opinions of Messrs. Lloyd and Salmon as to the symptoms and management of carcinomatous affections of the rectum. Mr. Lloyd maintained that these affections of the intestine were sometimes attended with pain and sometimes not, and that the patients were often materially benefited by the use of bougies; while Mr. Salmon affirmed, that the disease was always painful, and not to be mitigated by the employment of these means.

A variety of subjects came before the society at the next meeting. The president read a letter from Mr. Wansborough, of Fulham, describing the "original stomach-pump," a specimen of which accompanied the letter: the instrument Mr. W. stated to be of 25 years' standing.

Mr. Shearly exhibited, among other morbid specimens, a uterus, on the external surface of which the disease termed the "white tubercle" was in progress: there was also a small polypus near the os uteri. Mr. Pereira exhibited to the society a fine specimen of medullary sarcoma of the liver. The organ was so enlarged as to have occupied two-thirds of the abdomen. The other viscera were not involved in the disease. The progress of the disease, as far as it could be ascertained, was excellently detailed by Mr. Smith, a visitor. The symptoms were obscure.

Mr. Armstrong read an extract of a letter from Mr. Morgan, of the Bristol Infirmary, a corresponding member of the Society, describing a curious conformation of the œsophagus in a child, which lived 106 hours only, during which period every attempt to swallow food produced considerable distress. The œsophagus was found to be divided into two portions; the upper or pharyngæal extremity terminating in a *cul de sac* behind the bifurcation of the trachea; and the lower or ventricular extremity arising from the trachea at this point, between the bronchi, proceeding down the spine, and terminating, as usual, at the cardiac orifice of the stomach. Mr. Drysdale mentioned a somewhat similar malformation occurring in two children in one family. Mr. Callaway related a case of concussion of the

brain, which terminated fatally, after the restoration of the intellectual functions. The whole of the anterior lobes of each hemisphere of the brain was broken down, and mixed with effused blood. The remarkable feature of the case was, the return of consciousness, and the possession of this power for six days, under the disorganization of the brain described; and the questions particularly agitated were, Did this extent of mischief exist immediately after the concussion? What was the degree of laceration of the brain at that period? and, How could the recovery of the intellectual faculty be reconciled with so much injury of the brain?

The Society then adjourned to the 12th of January, 1829.

COOPER v. WAKLEY.

Notes of the Cause, Cooper v. Wakley—We understand that Mr. Cooper is about to publish a verbatim account of the above trial, from Mr. Gurney's short-hand notes.

ERRATA.

The Advertisement of Mr. B. Cooper's, in our last Number, ought to have been dated "*December* 15," instead of "*Oct.* 15." Also, for "*Mr. Elton*," the name of one of Mr. Cooper's witnesses, read "*Mr. Hilton*."

In our last leading article, *for* "can no friend of Wakley be named?" *read* "can the name of no friend of Wakley be mentioned?" The alteration is necessary, to connect it with the conclusion of the paragraph.

W. WILSON, Printer, 57, Skinner-Street, London.

FOOTNOTES:

[1] General thickening of all the valvular apparatus on both sides of the heart. (St. B. 33.)

[2] Andral, vol. iii. p. 411.

[3] Pl. 1, fig. 5.

[4] Opacity and thickening of the lining membrane on both sides of the heart, without unevenness or alteration of form. (St. B. 33).

Opacity and thickening in the valvular apparatus between the left auricle and ventricle, with shortening of the chordæ tendinæ, and such alteration of structure and form as to produce a considerable contraction of the aperture. (St. B. 26).

Such thickening of the whole valvular apparatus as to narrow the aperture of communication between the left auricle and ventricle to a mere slit. (St. B. 19).

Shrivelled and contracted state of the semilunar valves, in which a cartilaginous substance has entirely taken the place of the natural structure. (Hodgson, pl. 1, fig. 6).

[5] Osteum aortæ almost closed by ossified valves. (St. B. 15); (Hodgson, pl. 1, fig. 2).

[6] Fungus growing from the aortic valves, which are thickened and shortened. (St. B. 6, 20).

Fungus from the aortic valves, which are ulcerated. (St. B. 21).

Larger specimens of fungus, growing from the ulcerated edges of two of the valves of the aorta. (Hodgson, pl. 1, fig. 7).

[7] Dr. Baillie's plate.

[8] Polypous coagulum in the ventricle. (Coll. of Phys. 4, A. 16).

Coagulum firmly attached to the lining of the left auricle, with enlargement of its capacity. (St. B. 19).

[9] (St. B. 13).

[10] It occurs sometimes in the uterus.

[11] Memoir. de l'Acad. des Sciences. Morand, 1732. Morgagni, Epist. 27.

[12] Andral, v. 3, 466; Hodgson, plate 1, figure 7; St. B. 14.

[13] Corvisart, cap. 4. sect. 1.

[14] Active dilatation of the left ventricle. (College of Phys. 4, c. 10.)

The thickened walls of the left strikingly contrasted with the attenuated walls of the right ventricle. (St. B. 9).

Active dilatation of the left ventricle. Its capacity is strikingly contrasted with that of the right. (College of Phys. 4, c. 11.)

Passive dilatation of both ventricles, especially of the right. (St. B. 10).

[15] Morgagni, Epist. xxvii. 7.

[16] Ibid. 2.

[17] Ibid. 3.

Rupture of the left ventricle, with attenuation of its muscular structure. (St. B. 18).

Rupture, without attenuation, but with softening and looseness of texture in the muscular substance. (St. B. 22).

[18] Harvey, Exercit. altera.

[19] Rupture of the left ventricle without change in its structure. Bone deposited at the commencement of the aorta. (St. B. 27).

In turning over the Memoirs of the Royal Academy of Sciences, I find two cases of rupture of the heart, reported by M. Morand. They both occurred in the year 1730; and, strange to say, one was that of a Duchess of Brunswick, who was of the same family as George II. who also died of a ruptured heart. In the one, that of the Duchess, there was a manifest ulceration through the walls of the right ventricle, its structure being otherwise unimpaired; in the other, where the aperture was in the left ventricle, there was probably a simple rupture, for the flesh of the heart was so soft that the point of a probe would pass through it wherever it was rested. (Mem. de l'Acad. Roy. des Sciences, Ann. 1732).

[20] The trial as published in the Lancet occupied less than 21 pages—in the Gazette it extended, in the same type, to 33 pages and a half. In the Lancet, those parts, both of the evidence and speeches, which told most against Wakley, were omitted.

[21] [See passage in Italics, page 137.](#)

[22] The defendant, on leaving the court, was cheered by the populace in Palace Yard.—Lancet, Dec. 20.

[23] This we believe is false; nothing of the kind either occurred, or was stated at the trial.—E. G.

TRANSCRIBER'S NOTE

Obvious typographical errors and punctuation errors have been corrected after careful comparison with other occurrences within the text and consultation of external sources.

Except for those changes noted below, all misspellings in the text, and inconsistent or archaic usage, have been retained.

[Pg 116](#), 'immedate contact' replaced by 'immediate contact'.

[Pg 119](#), 'and attenutation in' replaced by 'and attenuation in'.

[Pg 126](#), 'Tue valuable paper' replaced by 'The valuable paper'.

[Pg 132](#), 'probably impossibly' replaced by 'probably impossible'.

*** END OF THE PROJECT GUTENBERG EBOOK THE LONDON
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